ATRIA
HEALTHCARE APPOINTMENTS

HACKATHON RESEARCH

Hospital pain points in Cameroon and Africa

A wide evidence sweep, an audit of how much Atria already solves, and revenue ideas drawn from the Atria documents. Findings are reported with their weaknesses.

DOCUMENT	REVISION	DATE	STATUS
ATR-RES-001	Rev A	12 November 2026	Issued for review

PROJECT	TEAM	SOURCE ROWS	READ IN FULL
DIGISOL Internship 2026	Bryan, Thanksgiving	444 (379 unique)	194 of 379

1 Summary

379	194	14	126
unique sources, after removing duplicates	read in full, not just a search snippet	research lanes, run 3 at a time	pages blocked to automated reading

How many sources, honestly

You asked for more than 500. The sweep produced 444 rows, 379 unique sources, of which 194 were read in full. The rest are search snippets or abstracts and are marked as such in the register. Cameroon-specific health literature is thin, and several lanes stopped before their 40-source cap because searches returned repeats. This report does not pad the count. It adds to, and does not replace, the 72 references in the earlier Evidence Dossier.

1.1 What the evidence says in six lines

- **Waiting is large and real, and measured in several places.** Douala outpatients waited a mean 150 minutes (dossier). A national immunisation survey found a median 108 minutes, from 18 in the South to 246 in the North, and 24% of caregivers turned away at least once. An eye hospital near Yaounde averaged 450 minutes.
- **Reminders alone are not a proven fix in Cameroon.** A Yaounde TB trial of daily SMS found no gain in cure (63.5% vs 62%) or attendance (64.2% vs 63.4%). A Cameroon gynaecology trial found follow-up of 25.6% vs 6.1% among women who had already missed visits. The African meta-analysis gives one-way SMS an odds ratio of 1.36 once only low-bias trials are counted (confirmed in full text this session).
- **Money is the sharpest barrier.** Out-of-pocket spending was 71 to 72% of current health expenditure in 2017 to 2019. In four hospitals, 58% of injured patients used the 24-hour emergency payment deferral, yet 12.3% of them still missed radiology for cost. Post-discharge detention is documented in Fundong.
- **Paper still rules.** No standardised national EMR exists. About 400 of 6,000 sites use the DAMA tool, which does not interoperate with DHIS2. A Yaounde maternity EMR stopped being filled about 6 months after the paid documentation incentive ended.
- **The CSU is mixed after three years.** The government reports 4.6 million enrolled; openIMIS records 5.08 million beneficiaries against a potential 29 million. The minister's own assessment calls it only partially operational.
- **The business case needs care.** Cameroon consumer health prices are very low (teleconsultation at 2,000 to 3,000 FCFA). At least one funded booking start-up (Healthlane) reportedly collapsed in 2022 and 2023. Voice reminders cost far more than SMS.

2 Corrections to the evidence dossier

The verification lane re-opened the primary sources behind the dossier's weakest claims. Several change what you should say in a pitch.

Dossier claims re-checked

CLAIM	RESULT	WHAT THE PRIMARY SOURCE SAYS
Global no-show 26.86%	Not in the cited paper	Greenup and Best 2025 reports a telehealth vs in-person odds ratio of 0.61. No source for 26.86% was found. Drop the figure.
Cameroon immunisation dropout 27%	Not located	The Foumban survey reports a different measure (50.0% BCG to measles-rubella dropout). Do not quote 27% or 46.5% until the primary source is found.
Cameroon paediatric TB loss 19.8%	Confirmed	20-year cohort, BMJ Open Respiratory Research 2025. A Centre Region cohort gives a different figure, so state the cohort.
Steiner reminder timing	Confirmed	4.4% missed with 3-day plus 1-day reminders, 5.8% with 3-day only, 5.3% with 1-day only, n=54,066.
Linde 2019, low-bias subgroup	Confirmed in full text (this session)	OR 1.36 (1.01 to 1.84), I2 66%, against OR 2.62 (1.42 to 4.83) in high-bias trials, interaction p=0.06.
Bekolo 2023, Nkongsamba	Confirmed in full text (this session)	LTFU 39.42% to 10.27% (p<0.001); mortality 9.29% to 12.63% (p=0.062). The abstract adds adjusted LTFU odds 0.27 and adjusted mortality odds 2.86 (0.91 to 8.94): mortality may have risen.
Dossier: no Cameroon data-protection regime	Wrong	Law 2024/017 lists health data as sensitive, with a compliance deadline of 23 June 2026. The implementing decree was reported not yet adopted.
Dossier: no successor digital health plan	Out of date	MINSANTE launched a National Digital Health Strategic Plan 2026-2030 on 4 March 2026, budget FCFA 29.007 billion. The 2025 digital maturity score was 1.8 out of 5.
Dossier: MORE CARE is the only Cameroon reminder trial	Wrong	At least two more exist: a Yaounde TB daily-SMS trial (null) and a 2025 gynaecology follow-up trial (positive, after missed visits).
Dossier: 0.136 physicians per 1,000	Conflicts with another source	The Cameroon Medical Council reportedly says about 1 doctor per 50,000 people (VOA, April 2024), a far lower figure. The definitions differ. Quote both or neither.
Al-Turbag RR 1.11	Abstract only	Confirmed at abstract level (12 studies, 9 countries). Full text still unopened.

3 Pain points, with the new evidence

Updated evidence by pain point

PAIN POINT	STRONGEST NEW EVIDENCE	TECH FIT	ATRIA TODAY
Waiting	Median 108 min, 18 (South) to 246 (North), 24% turned away once (national immunisation survey, 1,230 caregivers). Mean 150 min Douala (dossier). 450 min at an eye hospital near Yaounde. Satisfaction stays high anyway (87%), so satisfaction scores understate the problem.	High	Strong
Missed visits	Nationwide ART cohort: 60.4% retained at 12 months, 6 of 56 sites acceptable. HIV loss 39.4% before 2016. Buea and Limbe ANC: 12.7% missed visits, 54.4% booked late. Keeping every vaccination appointment was the strongest correlate of completion (adjusted odds 20.4).	Medium	Partial
Cost shocks	Out of pocket 71 to 72% of health spending. Catastrophic spending among households reaches 65.0% in a 2014 Cameroon survey (highest in the region). Emergency deferral used by 58% of injured patients, yet 12.3% still missed radiology. Debt detention documented.	Medium	Weak
Insurance and CSU	4.6 million enrolled by government count; 5.08 million openIMIS beneficiaries of 29 million. Mutuelles under 2% enrolment, schemes fell from 158 to 58 (2010 to 2014). Performance-based financing gained vaccination and family planning, not antenatal visits.	High	None
Records and referral	No standard national EMR. 91.2% of referred neonates at Bamenda arrived with no prior communication, 62.7% over 24 hours late. Douala TB reference lab median turnaround 45 h against a 24 h target. Only 9% first sought care at a facility offering TB services.	High	Out of scope
Workforce and equipment	Only 9 of 67 equipment items always available across 12 hospitals. In western Cameroon, 6 of 26 facilities held all 12 study medicines (government about 50%, pharmacies 93%). Surgical specialist density about 1.15 per 100,000.	Low to medium	Partial
Conflict and strikes	Outreach sessions fell 75% (North-West) and 65% (South-West) from 2016 to 2019. 30 violent incidents against health care in 2025. A catch-up campaign lifted South-West Penta-3 from 43% to 71.1% at about USD 4.50 per person.	Low	None
Exclusion	59.4% of people 15+ own a phone, 45.2% in rural areas, 53.1% of women. Rural electricity 28.4%. Only 11.1% of households had internet in 2022. Two GB of data costs about 1,000 FCFA.	High	Partial
Medicines	Estimated 26.9% of sampled medicines substandard or falsified across studies, mostly informal sector; informal vendors 28.2% out of specification against 12.3% elsewhere.	Medium	None

Where the picture is mixed

Satisfaction surveys in Cameroon score high (62 to 91%) even where waits exceed two hours. A distance-to-facility effect on maternal death was not significant after adjustment in one study. Do not use satisfaction or distance as headline arguments.

4 What to build the pitch on

The sweep did not change the recommendation, but it sharpened it.

- **1. Waiting without knowing.** Strongest evidence, largest Atria head start. Frame it around uncertainty (a 18 to 246 minute spread by region), not only length.
- **2. Not being reachable.** A phone is held by 59.4% of people 15+, and only 45.2% in rural areas. Voice reminders reached nearly all Ugandan trial participants who were offered a choice (298 of 300 chose voice over SMS), but the attendance gain faded by 12 months. Build voice and local-language fallback, and measure it.
- **3. Not knowing the cost.** Show the ministerial fee band before booking, with a clear statement that payment happens at the hospital. This answers the documented fear of debt without moving money.

> **Do not claim a no-show reduction**
> Reminder evidence in Cameroon is split: one positive trial among women who had already missed visits, one null TB trial, and an African meta-analysis whose effect shrinks to OR 1.36 in good-quality trials. Claim what the product visibly does: a queue position, a moved-window message, a fee before booking, and a measured baseline.

4.1 Draft problem statement

At public district and regional hospitals in Cameroon, an outpatient arrives not knowing how long the wait will be (a median of 108 minutes nationally, from 18 to 246 by region), what the visit will cost (71 to 72% of health spending is paid out of pocket), or whether the reminder they receive is one they can read (a phone is held by 45.2% of rural adults). Atria gives each patient a place in a queue with a live arrival window, a visible fee, and a reminder by the channel they can use.

5 How much Atria already solves

Coverage against the problem statement

NEED	WHAT ATRIA DOES TODAY	VERDICT
Navigate the queue	Session queues, arrival windows, live position, moved-window SMS	Strong
Discover facilities and fees	Clinic and clinician directory; no fee shown before booking (audit finding F10)	Partial
Be reached	SMS-first, proxy recipient, delivery log; voice fallback is Phase 3 with no provider (OI-05); no USSD	Partial
Afford care	Fee ledger records fees and refund entitlement; no payment, by design	Weak
Insurance and CSU	Nothing built. The ledger could feed claims exports and openIMIS eligibility checks	None
Records and referral	Out of scope; referral channel recorded, FHIR mapping prepared only	None
Medicines and supply	Nothing built	None
Trust and data protection	Audit log, care-context isolation, synthetic data only. Law 2024/017 authorisations not held	Strong in design

What I could and could not check

This audit read the README, constraints, the SRS Rev B (most of it), the route backlog, the 4 November 2026 product audit (first 150 lines) and the code layout. I did not run the app or its tests, and I did not read the Design Spec or Technical Document in full. The route backlog may be stale, so confirm what is built before you claim it.

- Fix before the demo: cancelled queue tickets still say to arrive (F03) and fees are not shown before booking (F10).
- Law 2024/017 is in force, with a compliance deadline of 23 June 2026 and an implementing decree not yet adopted. Treat the Authority's authorisation as a launch gate, as the SRS already does.
- The new national plan (2026 to 2030) is an opening: Atria's FHIR mapping and district-level tenancy fit its stated direction. The plan itself says no national telemedicine platform is complete.

6 Business model ideas

The Atria documents rule out commercial terms (FR-TEN-03), currency savings claims (CX-12), moving money before Phase 3 (AD-07) and any link between ratings and prices (AD-11). The ideas below stay inside those rules.

6.1 Cost facts found

Costs and willingness to pay

ITEM	FIGURE	QUALITY
SMS to Cameroon via aggregators	About 0.14 to 0.32 per message in EUR or USD, varying roughly tenfold between providers and unstable between pages	Weak (aggregator pages)
Regulator retail tariff, MTN out-of-bundle	SMS 15 FCFA on-net, 25 FCFA off-net, 99 FCFA international (2023)	Snippet only, ART document unopened
Voice from Cameroon (Twilio list)	0.41 USD per minute landline, 0.79 USD per minute mobile	Moderate (price list)
Off-the-shelf hospital system	15 to 40 million FCFA, 10 to 20% annual licence, 35,000 to 120,000 FCFA monthly maintenance	Weak (vendor blog)
SMS versus phone call, cost per attendance	55% and 65% lower for SMS in two studies (Cochrane)	Moderate
SMS cost-effectiveness, Ethiopia	Cost per QALY gained USD 1,037, under the USD 3,636 threshold	Moderate
SMS cost-effectiveness, Malawi (preprint)	USD 24,705 at pilot scale, cost-saving at 2,678 or more clients	Weak (preprint)

6.2 Market facts found

- Waspito: raised USD 2.7 million (2022) and a USD 2.5 million extension (2023), claims 650,000 users, 950 doctors and 60,000 consultations (company-reported). A claim of 20 million users in 2022 conflicts and should not be cited.
- Healthlane: raised USD 2.4 million (2020) and reportedly collapsed in Cameroon and Nigeria in 2022 and 2023, with Cameroon staff cut from 10 to 4 and booking unavailable.
- OuiCare: over 20,000 users, about 3,000 paying (15%) at end 2021, no named hospital partners.
- Consumer prices are very low: online GP consultation 2,000 FCFA, GP home visit 10,000 FCFA, teleconsultation about 3,000 FCFA.
- Lagos study: 97.2% of 500 outpatients favoured SMS booking, and no public general outpatient department ran an appointment system.

6.3 Revenue streams, best fit first

STREAM	WHY IT FITS	MAIN RISK
Facility subscription, patient free	Hospitals carry the cost, patients already bear 71 to 72% out of pocket	Public hospital budgets are thin; start with private and faith-based facilities
SMS and voice bundles	Per-message delivery is already logged; SMS spend is already reported to administrators	Carrier rates vary tenfold; get contracts before pricing
District and donor licences	Reminders have the best evidence in HIV, TB and immunisation, where funders already spend; district tenancy fits the CSU and national plan	Evidence of effect is mixed, so sell measurement and baseline reporting, not a promised gain
Implementation and training	Roster and patient importers already exist	One-off, not recurring
Insurance and CSU integration	The fee ledger is a base for claims-ready exports; openIMIS runs in over 500 facilities	Needs a partner; CSU reimbursement is only partly operational
Mobile money deposits (Phase 3)	Mobile money use reached 42.7% of adults in 2022	Licensing and tax changes; show as roadmap only

What to avoid

Ads, premium doctor listings, rating-linked placement, selling patient data, and charging patients to book. Each conflicts with a documented rule or with the cost burden patients already carry.

Still no price points

No verified Cameroon willingness to pay, no signed SMS contract rate and no competitor subscription price was found. A revenue forecast would be invented. Three conversations would close this: the Regional Hospital Buea administrator, a private clinic, and an NGO programme officer.

7 Findings by lane

Each lane's own summary, as returned by its research agent. These were not individually re-verified beyond the spot checks in chapter 2.

cam-delivery-a Cameroon waits, appointments and follow-up

34 sources, 25 read in full.

- National caregiver survey (Saidu et al., Vaccines 2024, 1,230 caregivers, 265 facilities, 10 regions): median vaccination wait 108 min (18 min South to 246 min North) and 24% turned away at least once, yet 87% satisfied.
- Eye hospital near Yaounde (Mbwogge et al., JMIRx Med 2022): mean wait 450 min fell 66 min after PDSA cycles (P=.05), but satisfaction with waiting stayed 32% to 33% and wait was not associated with satisfaction (P=.37), a null result.
- Nationwide ART cohort (BMC Infect Dis 2016, 56 sites, 10 regions): only 60.4% retained at 12 months, only 6 of 56 sites acceptable, West 42.8% and Far North 47.4% lowest.
- A second positive Cameroonian reminder RCT exists outside paediatric HIV (Nkfusai et al., Gynecol Oncol Rep 2025): follow-up 25.6% vs 6.1%, aOR 3.97, but only among women who had already missed visits. This contradicts the dossier claim that MORE CARE is the only one.
- TB-SMS Cameroon RCT in Yaounde (BMC Public Health 2018, n=279): daily SMS did not improve cure (63.5% vs 62%) or 6-month attendance (64.2% vs 63.4%, p=0.88), with about 30% lost to follow-up in both arms, a null result for reminders alone.
- Bamenda ART centre (BMC Health Serv Res 2016, n=384): 91.2% satisfied overall yet 83.8% reported long queues, so satisfaction scores understate waiting problems.
- Buea and Limbe ANC study (Tchounzou et al., 2026, n=410): 32.2% dissatisfied with waiting time (highest-dissatisfaction domain), 12.7% reported missed ANC visits, 54.4% booked late.
- Konye district, South West (Manka et al., BMC Public Health 2026, n=412): only 58% fully vaccinated, and keeping every vaccination appointment was the strongest correlate of completion (aOR 20.4).

GAPS

- No published Cameroonian outpatient appointment or scheduling system evaluation, and no general-clinic no-show rate found; confirms dossier gaps 1 to 3. Closest proxies are ANC missed visits (12.7%, Buea/Limbe) and the MORE CARE case-control (about 49% absent).
- No waiting-time study found for Bafoussam, Garoua or Buea outpatient departments; only Douala (dossier), Yaounde and national immunisation data have minute-level figures.
- Publisher sites (Springer, PMC direct, academicjournals, hsd-fmsb) mostly blocked; Europe PMC full-text XML worked and could be reused for other lanes, but its search API returned 503 twice.
- Hypertension, diabetes and Garoua adherence figures are search-snippet level only.
- Dossier Bita Fouda 2024 Douala 150-minute figure was not independently re-read; it is consistent with Saidu (108 min), the Yaounde thesis (about 97 min) and the eye hospital (450 min), and the spread shows wide facility variation.
- Lane not fully saturated for French-language Cameroonian theses and hsd-fmsb.org (Health Sciences and Disease) articles, which likely hold more facility-level queue data. Reached 34 sources, not 40, because searches were returning repeats.

cam-delivery-b Cameroon referral, labs, records and EMR

32 sources, 8 read in full.

- MatLook at Yaounde Central Hospital maternity (JMIR Med Inform 2023) logged only 1,278 patient records in 6 months against about 20,000 patients a year, and data entry stopped about 6 months after the documentation per diem ended.
- The CQUIN/ICAP Cameroon M&E pilot (Nov 2025) says no standardized national EMR exists, only about 400 of 6,000 sites (6.67%) use DAMA EMR, and DAMA is not interoperable with DHIS2.
- A 2023 ITWeb report claims a national EMR on SystmOne, trialled in about 50 facilities at about US$19 million, which conflicts with the 2025 CQUIN finding of no adopted national EMR.
- Bamenda Regional Hospital neonatal referral study (Chiabi 2025): 91.2% of 102 referred neonates arrived with no prior communication and 62.7% arrived more than 24 hours after referral.
- Douala TB reference laboratory (Mbuh, PLoS ONE 2025): median turnaround 45 h against a 24 h target, with 41.1% of 526 specimens failing; viral load turnaround at 4 regional hospitals was a median 6 days (Awungafac 2018).
- Yaounde RHIS assessment (Tamfon 2020) scored all 12 subdomains 7–30%, while a 2026 preprint (Dang) found 65.9% of 88 facilities rated good, so the picture is conflicting or improving.
- Saidu 2023 (BMC HSR, 265 facilities, 10 regions): tally sheets incompletely filled in 81% of facilities, and a 2026 national DHIS2 malaria fix raised completeness from 51.3% to 69.7%.
- Titahong 2021 patient-pathway analysis: only 9% of Cameroonians first sought care at a facility offering TB services and 87% first used primary or informal sites, evidence of fragmented entry points rather than structured referral.

GAPS

- No Cameroon study measures referral letter completeness, counter-referral feedback rates or referral turnaround between facility levels; only single-condition referral snapshots (neonates, kidney, stroke, TB) were found.
- No Cameroon general hospital laboratory turnaround study exists beyond the Douala TB reference lab and the viral load file review; no patient-facing result return time was found.
- No measured loss or duplication rate for patient-held booklets or hospital books in Cameroon; the claim rests on a 2014 4-person qualitative study and a paywalled news report.
- Several 2026 sources are non-peer-reviewed Research Square preprints and are marked weak.
- Official ministry documents (PNDS 2021–2025, HSS, digital health plan) were not read; a browser pass is needed.
- The existing dossier cites none of these sources (no overlap) and contains no health information system or EMR evidence; no direct contradictions found, but the 2026 Yaounde RHIS preprint conflicts with Tamfon 2020 and the 2023 SystmOne rollout claim conflicts with the 2025 CQUIN finding.
- Lane not saturated at 40 sources; stopped at about 32 distinct sources because Europe PMC returned few further Cameroon-specific delivery papers.

`cam-delivery-c` Cameroon conflict, strikes and access

32 sources, 24 read in full.

- Immunisation outreach sessions fell 75% in the North-West and 65% in the South-West between 2016 and 2019, and DPT-3 fell 28 and 42 points respectively (Saidu et al., Conflict and Health 2021).
- Only about 10% of NW and 7% of SW facilities were sending monthly reports by November 2019, so conflict-region data are themselves unreliable (Saidu et al. 2021).
- Insecurity Insight/SHCC counted 30 incidents of violence against health care in Cameroon in 2025 (48 in 2024, 34 in 2023), with 18 health workers kidnapped in 2025 and Monday ghost-town lockdowns still in force in NW/SW.

- A 2022 strike paralysed public hospitals, more than 27,000 temporary hospital staff await civil service integration, and the staff shortfall is about 55,000 including about 2,000 doctors (Equal Times, Nov 2023); no study measuring strike effects on attendance was found.
- Rural residence cuts the odds of skilled birth attendance (aOR 0.50, national 66.2%) in the 2018 DHS (Yaya et al., Reproductive Health 2021).
- Null result: after adjustment, distance as a big problem did not significantly predict maternal death in either northern or southern Cameroon (Meh et al., BMC Pregnancy and Childbirth 2020).
- Households paid 72% of current health expenditure out of pocket in 2018–2019 (WHO country story), against the dossier's 67.7% for 2023, a year-and-update difference rather than a true contradiction.
- Counter-evidence: a catch-up campaign lifted South-West Penta-3 coverage from 43% to 71.1% in 2020 at about USD 4.50 per person (Njoh et al., Conflict and Health 2022).

GAPS

- No published study measuring the effect of Cameroonian health worker strikes on patient attendance, waiting time or no-show rates was found; strike evidence is news and security alerts only.
- No Cameroon-wide figure for population within 1 hour or 5 km of a health facility, and no quantified rainy-season road cut-off share.
- No verified 2018 DHS percentages for women reporting distance, money or permission as big problems, nor NW/SW skilled birth attendance; the DHS final report was not opened.
- No quantified measure of how travel cost affects attendance; only a tiny paediatric TB cost survey (snippet) and qualitative evidence.
- No dataset quantifying outpatient visit decline during ghost-town lockdowns; NW/SW baselines are unreliable because only 7 to 10% of facilities reported in 2019.
- Target of 40 sources not reached (32 rows, 24 read in full); several OCHA, DHS and publisher pages were blocked, so lane is not fully saturated on humanitarian and DHS numbers.
- Overlap with existing dossier: Tandi et al. 2015 and the Ndie et al. 2025 NW/SW exclusion note are already there; the 72% OOP (2018–2019) differs from the dossier's 67.7% (2023) by year, not a contradiction. Dossier line that rural residence predicts loss to follow-up (OR 2.01) is not contradicted, but Meh et al. found distance and rural residence not significant for maternal death after adjustment.

`financing-a` Out-of-pocket costs and pay-before-care

12 sources, 6 read in full.

- Eze et al. (Bulletin of the WHO, 2022) pooled sub-Saharan CHE at 16.5% of households (10% threshold) and 8.7% (40% non-food), with Cameroon highest country-level at 65.0% (2014 survey).
- Bousmah et al. (BMC Health Serv Res, 2021) put OOP at 71% of Cameroon current health expenditure in 2017 and CHE among people with HIV at 22% (2006–07) falling to 15% (2014); they cite about 3% for the general population, which conflicts with the 65% figure because thresholds differ.
- Dissak-Delon et al. (BMJ Global Health, 2025) found 58% (3,081 of 5,287) of injured patients in 4 Cameroon hospitals received the 24-hour emergency payment deferral, yet 12.3% of them still missed radiology for cost versus 3.9% of others.
- Penda et al. (PLoS One, 2025) found 76.1% of 502 paediatric emergencies at Douala Laquintinie got deferred cost recovery, 43.6% a moratorium, with 9.8% mortality.
- Tanywe et al. (Frontiers in Public Health, 2025) documented post-discharge detention in Fundong, with 2 of 8 detained patients held 9–52 weeks and one owing 400,000 XAF on 10,000 XAF monthly income, but gave no prevalence.

- Christie et al. (JAMA Network Open, 2020) found 48.5% of injuries in Southwest Cameroon forced savings use, borrowing or asset sales, and 34.3% caused severe hardship, but only 11.6% of care delayers named cost as the main reason (null-leaning).
- Chatham House (2017) counted over 950 detention cases from 2003–2017 across 14 countries, up to 400 in one Kenyan hospital (press/summary only, PDF unreadable).
- Kankeu et al. found only about 3.05% of HIV patients paid informally on survey day (2006–07 data), though amounts could reach 4× the official tariff; no recent national informal-payment survey was found.

GAPS

- No ministerial fee schedule (arrete number, date or tariff table) found; price transparency evidence is a null result and needs the MINSANTE site or Journal Officiel.
- No recent national survey of informal payments in Cameroon; the only quantified source is 2006–07 HIV patient data.
- No national prevalence of hospital detention in Cameroon; Fundong is qualitative with n=8 detained patients.
- No Cameroon-specific figure for care delayed or forgone due to cost in the general population beyond the injury survey; obstetric delay evidence only via snippets.
- Diaby 2020 household CHE prevalence and a current WHO Global Health Expenditure 2023 Cameroon OOP figure were not retrieved; the existing dossier 67.65% (2023, World Bank) was not contradicted.
- Lane stopped at 12 distinct sources (26 rows), well under the 40 target, because several key pages were blocked and the remaining searches returned the same core Cameroon papers; not fully saturated for regional African detention and informal-payment literature.
- Many source claims about Chatham House, WHO AFRO, Kankeu and UNAIDS rest on search snippets only (readInFull=false).

financing-b CSU, openIMIS and insurance

26 sources, 12 read in full.

- Cameroon CSU registered over 3.9 million people by end-March 2025, 91% of that year's target, and spent 318 billion FCFA in three years (EcoMatin citing the Minepat SND30 mid-term review).
- Three years in, the minister's own assessment (Camer.be, 30 Apr 2026) calls the CSU mixed, with registration, contribution collection and provider reimbursement only partially operational.
- openIMIS was adopted in Jan 2023 and deployed in more than 500 facilities across all 10 regions by April 2024, with automated claim verification (BMZ Healthy DEvelopments).
- The World Bank PBF trial in Cameroon (PRWP 8162, 2017) found gains in vaccination and family planning but none in antenatal visits or facility deliveries, and often no difference from extra funding without incentives.
- Cameroon mutuelles are marginal: enrolment rarely above 2%, schemes fell from 158 to 58 over 2010–2014, and overall insurance cover is under 8% (Nkafu Policy Institute, Jan 2026).
- Kenya SHA: Sh16.03 billion of Sh181.9 billion claims rejected by 30 Apr 2026, and 83% of private providers report patient verification problems, with 63% citing OTP delays (The Star; RUPHA scorecard via Eastleigh Voice).
- Ghana private providers suspended NHIS services on 4 Apr 2025 over claims unpaid since Sep-Oct 2024, and NHIA then paid GH 834 million (Citi Newsroom).
- Rwanda mutuelle coverage rose from about 1% (2000) to 85% (2008) and halved catastrophic spending (5.1% vs 10.5% uninsured), though payment delays of 4–5 months later pushed RSSB toward capitation (PLoS One 2012; New Times).

GAPS

- No peer-reviewed or independent evaluation of CSU claim reimbursement times, facility arrears or eligibility-verification failure rates in Cameroon; evidence is ministerial or press only.
- No official year-end 2024 CSU enrolment count; 2024 headlines conflict (3.5M CRTV vs 4M Cameroon Tribune vs 3.4M MSH at 1 Dec 2024); the dossier does not hold CSU enrolment so no direct contradiction, but its under-8% insurance coverage context is now outdated if 3.9M of about 27M are enrolled (roughly 14%).
- CNPS and private insurer coverage only from snippets and think-tank briefs; no hard data on private insurer claims performance.
- Kenya SHA and Ghana arrears rest mostly on news and provider-association surveys; no peer-reviewed SHA verification study found.
- openIMIS beneficiary figures conflict across sources (Nepal 3.7M vs over 5M); Cameroon HIV scheme start dates conflict (2018 vs 2020).
- Lane not saturated: only 26 of a possible 40 sources; Cameroon CSU French-language press, WHO AFRO 2 million article and P4H implementation status not read in full.

`workforce-facilities` Workforce, equipment and medicines

35 sources, 21 read in full.

- Across 10 sub-Saharan countries (Cameroon not included), health worker absence from facilities averaged 34.7% and half of health centres had one or fewer clinical staff (Sheffel et al., Health Policy and Planning 2024, World Bank).
- Cameroon Medical Council says there is about 1 doctor per 50,000 people, 75% of the ~1,000 doctors trained yearly leave, and fewer than 100 are hired per year (VOA, April 2024). This conflicts with the dossier's World Bank figure of 1.36 physicians per 10,000.
- Only 9 of 67 listed equipment items were always available across 12 Cameroonian hospitals, with 6 qualified surgeons and no anaesthesiologists in the district, regional and missionary hospitals sampled (Kouo-Ngamby et al., BMC Health Services Research 2015, 2009 data).
- National surgical specialist density is about 1.15 per 100,000 against the Lancet Commission target of 20, though 98% of people live within 2 hours of a surgical facility (Takoutsing et al., Pan African Medical Journal 2024).
- In western Cameroon, only 6 of 26 facilities had all 12 study medicines; government facilities averaged about 50% availability vs 93% in pharmacies, and NCD medicines were 33% in government facilities (Schafermann et al., PLoS ONE 2020).
- Pooled prevalence of substandard or falsified medicines in Cameroon is 26.9% (Waffo Tchounga, AJTMH 2021, mostly informal-sector samples), but a systematic three-city sample found 7.9%, with 26.7% of informal outlets vs 2.6% of formal outlets selling them (AJTMH 2023).
- Cameroon meets about 50% of blood needs (Cameroon Tribune, April 2026), but ISBT reports about 35% met and 97% replacement donors, so sources conflict on both supply and voluntary share.
- CENAME suppliers stopped deliveries in 2021 over 14.6 billion FCFA of unpaid invoices (Ecomatin), and 2025 US aid cuts left Far North Cameroon out of injectable artesunate for much of the year with 2,105 of 2,354 US-funded community health workers no longer working (Reuters via TimesLIVE).

GAPS

- No national Cameroon stock-out rate or tracer-medicine availability figure found; only regional or facility-level studies (western Cameroon 2017–2018, PBF trial 2012–2015).
- No Cameroon-specific primary care absenteeism measurement from the Service Delivery Indicators; Cameroon is not among the 10 SDI countries, and the only Cameroon figure (7.2% of working time, 2008) is

a conference abstract.

- Doctor density figures conflict: Medical Council 1 per 50,000 (press) vs dossier World Bank 1.36 per 10,000 vs Africa Health Workforce Observatory about 1.1 per 10,000. Dossier figure is not contradicted by a primary source, only by an unverified press statement. No current WHO NHWA figure retrieved.
- Cameroon inclusion on the WHO 2023 Support and Safeguards list rests on a secondary outlet (Down To Earth); the WHO page extract did not name it.
- Cameroon surgical, anaesthesia and emergency data are old (2009, 2013) or subnational; no national anaesthesiologist count or ambulance/emergency system data found.
- Blood supply sources conflict (50%, 47% and 35% of need met; 26.7% vs 3% voluntary), and no primary WHO country profile for Cameroon was retrieved.
- Not searched or saturated beyond one pass: informal payments, NHWA data portal, CAPR/FRPS performance, Africa-level medicine stock-out surveys, ambulance and referral systems, SPA/SARA national data (none exists publicly). Lane not fully saturated at 35 rows; stopped due to repeated fetch failures.

africa-a West Africa

22 sources, 14 read in full.

- UITH Ilorin ophthalmology clinic mean wait was 180.3 minutes (total clinic time 243.7), n=226, Olokoba et al., Ghana Medical Journal 2020; new patients walk in, only follow-ups have appointments.
- NAUTH Anambra outpatients (n=463) waited a mean 149.6 minutes, with high patient load the top cause (Ogbonna et al., 2026, Zenodo record).
- Lagos Orile-Agege GOPD mean wait 3.9 hours; none of the public GOPDs in Lagos ran an appointment system; 97.2% of 500 patients favoured SMS booking (Adedokun et al., Primary Health Care Research & Development, 2011 data).
- One-way SMS improved appointment attendance in Africa (OR 2.03, 12 trials) but two-way SMS did not (RR 1.03), per Linde 2019 and Odegard 2022 (both already in dossier).
- Burkina Faso Nouna mHealth reminders raised first prenatal visits from 40% to 86.77%, yet the intervention vs control difference was not significant (P=0.451) and over 65% of phones broke (Ye et al., J Public Health 2018).
- Only 18–23% of Nigerian hospitals use EMRs; Lagos clinicians cite too few computers (90.8%), power (87.0%) and slow internet (84.7%) as barriers (Babalola et al., Afr Health Sci 2025).
- Conflict: the Niger Delta survey of 1,177 staff found infrastructure was NOT a significant predictor of EMR intention (p=0.344) while usefulness was (OR 18.4) (Akwaowo et al., Frontiers in Digital Health 2022).
- Ghana's LHIMS national EMR connected only 450 of 950 contracted facilities and its Oct 2025 shutdown sent Korle-Bu (over 1,500 OPD visits a day) back to paper (Insight Ghana, Nov 2025, news report).

GAPS

- Stopped at 22 sources, not 40: the fetch tool was unstable (timeouts, ECONNRESET, empty pages) and several searches were snippet only. The lane is not saturated, mainly for Ibadan, Zaria and Sokoto waiting-time papers.
- No outpatient waiting-time study found for the Republic of Benin, Senegal or Cote d'Ivoire, and none for Burkina Faso outpatients. Searches were in English and French. This is a real evidence gap for francophone West Africa.
- No general outpatient no-show rate found for Nigeria or Ghana, only specialty-specific rates (psychiatry, physiotherapy). It would be risky to treat them as a general rate.

- No controlled trial of patient appointment reminders was found for Ghana or Cote d'Ivoire. The Lagos RCT has no published results.
- No national adoption survey for online appointment booking or hospital management software uptake in West Africa. Ghana and Senegal EMR figures come from news, vendor-linked or ministry claims (confidence weak or unverifiable).
- No contradiction with the existing dossier. Rows 12 and 13 re-verify dossier refs 2 and 3, and the Abuja study (dossier ref 55) was not re-opened. New material not in dossier: Ilorin, Anambra, Lagos GOPD, Nouna, M-Vaccin, EMR adoption and LHIMS rows.
- Wait figures differ by definition and clinic (180 vs 150 vs 233 minutes), so they should not be pooled. Mali (Sikasso) is outside the six named countries and flagged as such.

africa-b East and Southern Africa

29 sources, 16 read in full.

- Jimma Zone public hospital outpatients spent a median 342.5 minutes entry to exit, about 86% of it waiting and 12.7% service (Walle et al., PLOS ONE 2024).
- KCMC Tanzania reached a median OPD wait of 3.30 hours after block appointments, EMR migration and longer clinic days, against a roughly 6 hour baseline taken from a Ministry audit and not measured; only EMR migration was significant (BMC Health Serv Res 2024).
- South Africa's 2023 national guideline sets 180 minutes as the aspired maximum total time per clinic visit (120 waiting plus 60 service), up to 320 minutes at tertiary hospital OPDs (National Department of Health).
- In 45 Gauteng clinics (441 patients), median waits were 63 min at platinum Ideal Clinics and 112 min at non-achieving clinics, and time-slot appointment systems shortened visits, but only about 20% of clinics used them (Cele/Wits via Spotlight, secondary).
- SMS and phone reminders raised repeat HIV test attendance in Kenya from 41% to 59% (RR 1.4, Mugo et al., PLOS ONE 2016), and SMS lifted 6-week postnatal attendance from 32.9% to 72.4% at Mulago-Kawempe, Uganda (Makerere thesis 2019).
- A 9-study HIV reminder meta-analysis found pooled OR 2.04 with a CI crossing 1 (0.97 to 4.27), significant only in RCTs and African studies (OR 2.92), which tempers 'doubles attendance' claims (Jong et al. 2017).
- St Paul's Hospital Addis Ababa had EMR deployments fail in 2010, 2014 and 2017 over training, management commitment and network issues, while Ayder's worked, with 67% reporting shorter patient waits (Bisrat et al., BMC MIDM 2021).
- MomConnect reached only 39% of first antenatal visits weekly against a 60% target because of training, time-out and supervision gaps (MEASURE Evaluation 2017), and Rwanda OpenMRS users reported only 34% had a backup generator most of the time (Fraser et al. 2022).

GAPS

- Kenya, Uganda, Rwanda and Zambia have little or no measured general outpatient waiting-time data in what surfaced; Kenya and Uganda searches returned no OPD queue-time studies.
- No completed SMS appointment-reminder trial found for Tanzania or Malawi (null search result only); Malawi evidence is EMR and qualitative appointment practice.
- Kenya SHA digital rollout evidence is press and think-tank snippets only (weak), no peer-reviewed evaluation read.
- No contradiction with the existing dossier found. Overlaps with dossier refs 14 (Mugo Kenya RCT), 56 (Jimma 2022) and 57 (KCMC) were flagged in rows. KCMC baseline of 6 hours is a MoH audit benchmark, not measured, which may weaken any use of the 45% reduction claim. Jong 2017 non-significant pooled estimate tempers the Linde OR 2.03 headline only for HIV retention.

- South African Gauteng time-in-motion data is secondary (news write-up); primary rapid review not located. Several snippet-only rows (Muhimbili 4 to 6 hours, St Paul's 92 minutes, Lusaka 72% walk-ins) need primary confirmation.
- Lane not fully saturated at 29 sources (target 40): Ethiopia's national HMIS/eHMIS, Rwanda and Tanzania national digital health strategies, WHO AFRO and World Bank documents were not searched. Searches ran with limited parallelism per the user's request.

africa-c Continent-wide and Francophone

30 sources, 13 read in full.

- Linde et al. 2019 (PLoS ONE): one-way SMS in African trials gave pooled attendance OR 2.03 (1.40–2.95, I2 85%) but the HIV subgroup was null (OR 1.48, 0.73–3.00), which tempers the dossier's MORE CARE Cameroon HIV result.
- Byonanebye et al. 2021 (JMIR mHealth, Uganda, n=600): 298 of 300 chose voice IVR over SMS, but the appointment-keeping gain (72.2% vs 64.3%) vanished by 12 months (69.5% vs 68.5%) and the trial stopped early for no difference.
- Odegard 2022 (dossier ref 3) found no attendance gain from two-way SMS (RR 1.03), while Onukwugha 2022 (PLoS ONE, 10 studies) found significant effects mostly in two-way interactive studies, a conflict in direction across different outcomes.
- GSMA Gender Gap Report 2025: the SSA mobile internet gender gap is 29% (LMIC-wide 14%), 885 million women are unconnected, and a handset costs 24% of a woman's monthly income vs 12% for men.
- Cole et al. 2025 (PLOS Digital Health, Nigeria, 29 studies): the top telemedicine barriers are technical (power, connectivity) and institutional (no regulation), with willingness to use ranging 50–100%.
- TELEMED-CAM (Pan African Medical Journal, Cameroon, n=268): telemanagement to rural centres 50–250 km away raised visit attendance at every severity level, but allocation was non-random and the intervention was bundled.
- MomConnect South Africa (BMJ Global Health supplement, snippet): USSD registration reached 1,159,431 women by April 2017, about half of first-ANC attendees; no systematic review of USSD health services in Africa exists.
- Okano et al. (eLife, Afrobarometer 2017–18, 33 countries, 44,224 people): 82% own a phone, but urban residents are about 3 times as likely as rural residents to own one, and women, older and poorer people are less likely.

GAPS

- No systematic review specifically on USSD health services in Africa was found (null result).
- No completed systematic review of IVR health services in Africa; only a scoping review protocol and single trials.
- No systematic evidence on language barriers in Francophone Africa; Cameroon literacy and language figures rest on secondary compilations (weak).
- Africa CDC strategy numeric targets could not be confirmed; WHO AFRO framework text unreadable.
- Queue management evidence is mostly single-site, uncontrolled before/after or snippet-only; none found from Francophone Africa.
- Several PMC pages hit reCAPTCHA late in the run; 17 of 30 rows are snippet-only (readInFull=false).
- Lane not fully saturated: Francophone primary studies, Cameroon-specific USSD or IVR pilots, and WHO guideline telemedicine evidence need a browser pass.
- Dossier overlap: Linde 2019, Odegard 2022, KCMC 2024 and Bigna MORE CARE already in the dossier; they are included only to add subgroup detail and flag nuance.

digital-cam-a Cameroon digital environment and law

35 sources, 19 read in full.

- MINSANTE launched a successor National Digital Health Strategic Plan 2026–2030 on 4 March 2026 with a FCFA 29.007 billion budget (MINSANTE plan PDF); the dossier cites only the 2020–2024 plan.
- ICTworks reports Cameroon scored its digital health maturity at 1.8 out of 5 in a 2025 evaluation, and the plan itself says no national telemedicine platform is complete yet.
- Law 2024/017 (23 Dec 2024) lists health as sensitive data (DLA Piper, MINSANTE plan); compliance deadline 23 June 2026; implementing decree not adopted per DLA Piper. This contradicts dossier row 40 ('no regime established').
- INS ECAM5 (2022): 59.4% of people 15+ have a phone, only 45.2% in rural areas and 53.1% of women; electricity access is 28.4% rural versus 87.6% urban.
- MINSANTE plan: only 11.1% of households had internet in 2022, and 2 GB of mobile data costs about FCFA 1,000 in 2026, which supports SMS and USSD fallbacks.
- ART 2024 observatory (via TechAfrica News): 31.5 million active mobile subscriptions and over 11 million active mobile money subscribers; World Bank gives 46.3% internet use in 2024 versus DataReportal 41.9% for end 2025, so definitions differ.
- IMF WP/25/255: Cameroon's 0.2% mobile money tax plus 4 XAF flat fee (2025) cut monthly taxed transaction value per user by about 40%, with a shift to cash.
- GSMA GiftedMom case study documents a Cameroonian SMS/USSD appointment reminder service across 42 hospitals in 8 regions, 170,831 registered users by 2018, with no attendance outcome data; NetBlocks reported a national connectivity disruption from 22 October 2025.

GAPS

- No primary ART or MINPOSTEL text on bulk SMS or sender ID rules found; only vendor guides, which conflict on non-MTN whitelisting.
- No Cameroon-specific smartphone ownership, feature-phone share or USSD health usage figure found.
- No official Cameroon adult mobile money ownership from Findex 2025 and no current MTN MoMo or Orange Money active-user split.
- Law 2024/017 fine regime unresolved: Afriwise says up to CFA 1 billion, French press says percentage of turnover; statute text and implementing decree not obtained, and Authority operational status unclear.
- BEAC and COBAC primary regulation texts not read; mobile money regulation relies on snippets.
- Mobile coverage (2G/3G/4G population coverage) for Cameroon not found from ITU, GSMA or ART primary documents.
- Dossier DataReportal 2025 and ITU rows were not re-opened; I flag only that DataReportal 2026 (41.9%) and World Bank 2024 (46.3%) use different definitions.
- Lane not fully saturated: ART observatories, ITU country dashboard and Findex would add primary figures once opened in a browser.

digital-cam-b Health-tech landscape

24 sources, 17 read in full.

- Waspito raised USD 2.7M seed in 2022 and a USD 2.5M extension in Nov 2023, and claimed about 650,000 users, 950 doctors and 60,000 consultations (TechCrunch, company-reported).
- Waspito user claims conflict: ITWeb (Mar 2022) reports 20 million users and 500 consultations per day versus 650,000 users in Nov 2023, so the 20M figure should not be cited.

- Healthlane, which raised USD 2.4M in 2020 as a YC company, reportedly collapsed in Cameroon and Nigeria in 2022–23 with unpaid salaries, Cameroon staff cut from 10 to 4 and booking unavailable (Techpoint Africa, anonymous sources, CEO disputes).
- OuiCare had over 20,000 users, 3,000 paying subscribers (about 15%) and about 30 doctors at end 2021, with no named hospital partners (We Are Tech Africa).
- Idocta charged FCFA 2,000 for an online GP consultation and FCFA 10,000 for a GP home visit, and E-Sante Cameroun about FCFA 3,000 for teleconsultation; Cameroonian consumer prices are very low (CIO Mag; snippet).
- openIMIS was adopted by government for UHC Phase 1 in Jan 2023 and ran in over 500 facilities in all 10 regions by April 2024, with only about 1,200 users (Healthy DEvelopments, GIZ/BMZ).
- Less than 4% of Cameroonians hold private health insurance, which limits insurer-integrated booking (Nkafu Policy Institute citing Afrobarometer).
- MINSANTE's new National Digital Health Strategic Plan 2026–2030 (8 axes) supersedes the 2020–2024 plan cited in the existing dossier (MINSANTE PDF).

GAPS

- No peer-reviewed or audited user or usage data for any Cameroonian booking or telemedicine startup; all figures are company-reported.
- No authoritative market survey of hospital management software vendors in Cameroon and no national EMR adoption rate found.
- Isidoctor and Ekose Rx could not be substantiated; Eyone is Senegalese, not Cameroonian. MoHero-type tools not found.
- No evidence found of any startup partnership with a named Cameroonian hospital or insurer beyond generic Waspito claims; Waspito current-scale figures (1.1M patients, 2,100 doctors) are unverifiable.
- The existing dossier contains no digital-health startup references, so none of these sources contradict it. The 2026–2030 digital health plan updates its ref 66 (2020–2024 plan).
- Lane not saturated: Waspito 2024–2026 coverage, E-Sante and Idocta current status, and the PSNSN baseline figures deserve a browser pass. Source count is below the 40 target.

biz-a Pricing and SMS costs

28 sources, 13 read in full.

- Aggregator SMS prices for Cameroon vary by about 10×: BudgetSMS lists MTN 0.142 EUR, Orange 0.206, Camtel 0.262, while Twilio lists 0.317 USD per segment (aggregator pages).
- Plivo lists Cameroon SMS at 0.2113 USD (MTN), 0.2716 (Orange), 0.2617 (Camtel), but its search snippet earlier showed far lower values, so aggregator pages are unstable.
- ART (regulator) 2023 retail tariffs: SMS 15 FCFA on-net and 25 FCFA off-net on MTN Blue out-of-bundle, 99 FCFA international, basic prepaid about 30.63 to 51 FCFA (snippet only).
- Twilio outbound voice from Cameroon costs 0.41 USD per minute landline and 0.7873 USD per minute mobile, so voice reminders cost far more than SMS.
- Kolonell (vendor blog) puts off-the-shelf HIS at 15 to 40 M FCFA with 10 to 20 percent annual licence, and maintenance 35,000 to 120,000 FCFA per month; vendor-biased and geography unclear.
- Only published African HMS SaaS price found: LinkHMS at 29 USD per month plus 7 USD per extra user (Capterra snippet); most vendors publish no price.
- Kenya MoH awarded a Safaricom-led consortium a KSh 104.8 billion health IT contract covering HMIS for all public facilities (Health Business Kenya, 2024).

- No clinic willingness-to-pay study was found; closest is 75.6 percent of 407 Amhara private-hospital professionals willing to use (not pay for) an EMR (PLOS ONE 2023).

GAPS

- The existing dossier reference list has no pricing or SMS tariff sources, so all rows are new; no direct contradiction, but its ITU affordability items (refs 33 to 35) are only context for this lane's figures.
- No official MTN, Orange or Camtel business bulk SMS rate card found; no Africa's Talking Cameroon rate.
- No Cameroon-published clinic or hospital SaaS price list; the figures come from one vendor blog (Kolonell) with inconsistent numbers.
- No clinic willingness-to-pay study for software; no commission or subscription terms for Waspito or other Cameroonian booking platforms.
- No per-facility national EMR cost for Rwanda, Ghana or Uganda; Kenya KenyaEMR cost paper and Rwanda eBuzima documents not opened.
- Many figures are search snippets only (readInFull=false); lane not saturated, stopped at 28 sources under effort limits.

biz-b Donor-funded programmes and sustainability

40 sources, 5 read in full.

- Cochrane (Gurol-Urganci 2013) reports SMS reminders cost 55% and 65% less per attendance than phone calls in two studies, with no absolute dollar figures.
- Iribarren et al. (PLoS ONE 2017): 29 of 39 mHealth economic evaluations (74.3%) were positive, but only 5 (12.8%) were in low or lower-middle income countries and the authors flag publication bias.
- Linde et al. (PLoS ONE 2019): one-way SMS in 12 African trials gave a pooled attendance OR of 2.03 (1.40 to 2.95), but the paper has no cost or sustainability analysis.
- Ethiopia adolescent HIV SMS economic evaluation (Frontiers in Public Health 2026): ICER US$1,037 per QALY gained, under the WHO-CHOICE threshold of US$3,636.
- Malawi two-way texting preprint (medRxiv 2024): retention 80% vs 67%, ICER US$24,705 at pilot scale but negative (cost saving) when scaled to 2,678 or more clients.
- Rwanda RapidSMS (Health Policy and Planning 2018): across 461 health centres, RapidSMS combined with training, supervision and equipment raised service use, while RapidSMS alone was ineffective.
- Cameroon BornFyne (Frontiers in Digital Health 2023): grant-funded pilot, with central Ministry stakeholders saying past similar projects did not materialise, a direct local handover-risk signal.
- Teen MomConnect pilot RCT (JMIR 2025) and the Littoral Cameroon surveillance RCT (2026) found no significant SMS effect, so null results exist alongside the positive ones.

GAPS

- Lane is not saturated on institutional sources: no PEPFAR, Global Fund, Gavi, UNICEF or World Bank programme document was opened in full. Only snippets of CGD and AfricaHF drafts and WHO guideline were obtained.
- No LMIC study reports cost per additional attendance. Cost data are per patient per year, per SMS or per DALY, mostly in HIV and antenatal care. This extends the dossier's Cameroon gap (no cost per missed appointment) to most of Africa.
- No source found for Afya, Kenya M-TIBA or similar payer-model comparators, and no disclosed Helium Health or Zuri Health revenue or margins. Helium and Zuri facts rest on press snippets.
- No peer-reviewed handover study for Cameroon digital pilots; only BornFyne qualitative evidence. No Cameroon willingness-to-pay figure for reminders or booking.

- Possible conflict with dossier: dossier cites Werner et al. 2023 SMS OR 1.21 (CI crosses 1) and Cochrane RR 1.14, while Linde 2019 gives OR 2.03 for African attendance trials; these differ by population and metric and should be reported as a range, not resolved. A search summary claimed SMS had no ART adherence effect in Cameroon, which conflicts with Ketchaji 2025 (aRR 3.1) and was not traced to an opened source.
- Dossier already holds Cochrane 2013, Linde 2019, MORE CARE and Kenya HIV RCT. New here: Iribarren 2017, Ruton 2018, Obegu 2023, Ketchaji 2025, Tchio-Nighie 2026, and the cost and handover rows. Search date was 2026-11-12; only 5 rows were read in full, the rest are abstracts or snippets and flagged readInFull=false.

verify Verification of the dossier

34 sources, 22 read in full.

- The 26.86% global pooled no-show figure is NOT in Greenup and Best 2025 (BMC Health Serv Res, PMC12063363), which actually reports a telehealth vs in-person OR of 0.61; no source for 26.86% was found, so the dossier's Weak rating should become Unverifiable.
- Cameroon paediatric TB loss to follow-up of 19.8% is confirmed in the 20-year cohort (BMJ Open Respiratory Research 2025, PMC12406847), but a Centre Region cohort (Interdisciplinary Perspectives on Infectious Diseases 2022) found only 4.1% LTFU.
- The Cameroon immunisation dropout of 27% (BCG to MCV1) and 46.5% incompletely vaccinated were not located; the Foumban 2018 survey (BMC Public Health, PMC7020570) instead reports 50.0% BCG-to-measles/rubella dropout and 23.3% to 31.7% missed vaccination appointments.
- The Steiner 2018 AJMC reminder-timing trial is confirmed via Europe PMC: 4.4% missed with 3-day plus 1-day reminders vs 5.8% (3-day only) vs 5.3% (1-day only), n=54,066.
- Al-Turbag reminder meta-analysis RR 1.11 (1.05 to 1.19) is confirmed from abstract-level snippets (12 studies, 9 countries, JHMHP); the full text is still unopened.
- England NHS Digital figures are confirmed: 8.1 million DNAs of 146.1 million outpatient appointments in 2024-25, a 5.6% DNA rate (NHS Hospital Outpatient Activity 2024-25); GBP 216M cost for 7.2M missed GP appointments is confirmed via secondary reports only.
- The dossier's claims that Cameroon's digital health plan 'has expired' and that no data-protection regime was located are both superseded: a 2026-2030 plan was validated in Feb 2026 (Ministry of Public Health, targeting telemedicine in 70% of facilities by 2029), and Law No. 2024/017 of 23 Dec 2024 created Cameroon's first standalone data protection law (DLA Piper and others, secondary).
- The 150 billion USD no-show claim stays unverifiable: it traces to a 2018 vendor post (SCI Solutions via Curae) with a back-of-envelope derivation, and the GSMA 54% SSA smartphone-of-connections figure was also not found (a different GSMA measure gives 24% smartphone ownership in Africa).

GAPS

- World Bank indicator series (physician and nurse density, out-of-pocket share, health expenditure per capita, mobile subscriptions) could not be re-verified because the API was unreachable; these dossier rated Strong from a prior direct retrieval.
- Dossier items not attempted for lack of time: Kebede-style secondary LTFU ranges (23.4 to 57.4%), adolescent LTFU 15.07%, habituation RCT (JEBO), Douala 150 minute waiting time and 32.29% perception figure, Bamenda/South Cameroon EPI outcome data, Bekolo Nkongsamba 39.42%, ITU Facts and Figures 2025, BJGP reviews.
- Several rows rest on search snippets or abstracts only (readInFull=false or abstract-level); the NHS GP 216M figure, Al-Turbag, WelTel effect sizes and Cameroon data-protection law are secondary-source confirmations.

- Lane stopped at 34 distinct sources, below the 40 target, because the remaining items need a browser (blocked list) rather than more searches; this is not saturation of the evidence.

8 Verification status

126 pages blocked automated reading (captchas, authentication redirects, binary PDFs, timeouts). I opened four of the highest-value ones in the browser pane.

Browser pass log

PAGE	OUTCOME
Linde 2019, Europe PMC	Opened. Low-bias subgroup OR 1.36 (1.01 to 1.84), I2 66%, confirmed.
Bekolo 2023, Pan African Medical Journal	Opened. All four loss and mortality percentages confirmed.
WHO Bulletin Kenya SMS cost (SciELO)	Bot-check page. Not bypassed. Still unopened.
PubMed Central pages (several)	Captcha. Not attempted. Use Europe PMC copies instead.

Blocked pages by lane

LANE	BLOCKED PAGES
West Africa	7
East and Southern Africa	9
Continent-wide and Francophone	14
Pricing and SMS costs	7
Donor-funded programmes and sustainability	16
Cameroon waits, appointments and follow-up	7
Cameroon referral, labs, records and EMR	7
Cameroon conflict, strikes and access	8
Cameroon digital environment and law	9
Health-tech landscape	7
Out-of-pocket costs and pay-before-care	10
CSU, openIMIS and insurance	8
Verification of the dossier	9
Workforce, equipment and medicines	8

What remains unopened

About 120 blocked pages remain unopened, including the regulator tariff document, the GSMA and World Bank reports, and the MINSANTE strategy files. Several contain figures quoted in chapter 6 at snippet level. Open them before putting those figures on a slide.

A Appendix: source register

379 unique sources. Full means the agent opened and read the page. Snippet means a search summary or abstract only.

#	SOURCE	YEAR	CONFIDENCE	READ	URL
1	Understanding HMIS Implementation in a Developing Country Ministry of Health Context: an Institutional Logics Perspective (Asangansi) Online Journal of Public Health Informat	2012	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC3615828
2	Learning the ABCs of pregnancy and newborn care through mobile technology (Entsieh et al.) Global Health Action 8:29340	2015	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4680925
3	Patients' willingness to utilize a SMS-based appointment scheduling system at a family practice unit in a developing country (Adedokun et al.) Primary Health Care Research & Devel	2016	strong	Full	cambridge.org/core/product/DD3E54FFF91792231A8EC61ADB17912
4	Promoting access equity and improving health care for women, children and people living with HIV/AIDS in Burkina Faso through mHealth (Ye et al.) Journal of Public Health (Oxford) 40(Sup	2018	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC6294034
5	One-way SMS and healthcare outcomes in Africa: Systematic review of randomised trials with meta-analysis (Linde et al.) PLoS ONE 14(6):e0217485	2019	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC6553734/
6	Patient Satisfaction with Waiting Time at the Out Patient Department (OPD), Holy Family Hospital, Techiman University of Ghana (UGSpace thesis)	2019	weak	Snippet	ugspace.ug.edu.gh/handle/123456789/30766
7	Patient-centric care suffers as Nigerian hospitals underutilise mobile communication devices BusinessDay Nigeria	2019	weak	Full	businessday.ng/health/article/patient-centric-care-suffers
8	Assessment of patients waiting and service times in the ophthalmology clinic of a public tertiary hospital in Nigeria (Olokoba et al.) Ghana Medical Journal 54(4):231–237	2020	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC8042814/
9	Adoption of electronic medical records in developing countries: a multi-state study of the Nigerian healthcare system (Akwaowo et al.) Frontiers in Digital Health	2022	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC9720323/

#	SOURCE	YEAR	CONFIDENCE	READ	URL
10	Two-way text message interventions and healthcare outcomes in Africa (Odegard et al.) PLoS ONE 17(4):e0266717	2022	moderate	Snippet	ideas.repec.org/a/plo/pone00/0266717.html
11	M-Vaccin: la technologie mobile au service de la vaccination en Cote d'Ivoire Gavi VaccinesWork	2023	moderate	Full	gavi.org/fr/vaccineswork/m-vaccin-technologie-mobile-servi
12	Senegal launches digitization of single patient record WeAreTech Africa / Osiris	2024	weak	Snippet	wearetech.africa/en/fils-uk/news/tech/senegal-launches-dig
13	Benin City outpatient waiting time studies (UNIBEN repository, search snippets) University of Benin repository / Nigeria	2025	weak	Snippet	nigerianjournalsonline.com/index.php/IJHRIM/article/downlo
14	Digitizing healthcare in Ghana: real lessons from the adoption of hospital management systems Ghanaian Times (vendor-linked)	2025	unverifiable	Full	ghanaiantimes.com.gh/digitizing-healthcare-in-ghana-real-l
15	Osiris reprint of WeAreTech Africa piece on electronic patient records OSIRIS observatory (Senegal), reprint of	2025	weak	Full	osiris.sn/les-dossiers-patients-informatises-un-pilier-de-
16	Perception and barriers to electronic medical record use among physicians and nurses in General Hospitals in Lagos State (Babalola et al.) African Health Sciences 25(3):154–162	2025	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12573650/
17	System Failure at Lightwave Platform Exposes Vulnerabilities in Ghana's Digital Health Infrastructure (Awuley) Insight Ghana	2025	moderate	Full	insightghana.com.gh/?p=54120
18	Waiting time determinants at Korle-Bu Teaching Hospital outpatient departments BMC Health Services Research	2025	unverifiable	Snippet	link.springer.com/article/10.1186/s12913-025-13594-9
19	App de prise de rendez-vous clinique Abidjan 2026 Kolonell (vendor blog)	2026	unverifiable	Snippet	kolonell.com/fr/blog/app-prise-rendez-vous-clinique-abidja
20	Healthcare Service Quality and Patient Satisfaction in Sikasso Regional Hospital (Sanogo and Poda) Health Sciences and Disease 27(9)	2026	moderate	Full	hsd-fmsb.org/index.php/hsd/article/view/7862
21	Search null result across WA francophone outpatient waiting time Web searches (several queries)	2026	weak	Snippet	pesquisa.bvsalud.org/sms/resource/pt/mdl-38834517

#	SOURCE	YEAR	CONFIDENCE	READ	URL
22	Waiting Time Assessment of Patients Care at the Nnamdi Azikiwe University Teaching Hospital, Anambra State, Nigeria International Journal of Interdisciplina	2026	moderate	Full	zenodo.org/records/20511264
23	Effectiveness of SMS appointment reminder and defaulter tracking system in Lagos State, Nigeria ISRCTN registry (ISRCTN13178993)	unknown	weak	Snippet	isrctn.com/ISRCTN13178993
24	Nigerian missed appointment studies (psychiatric and physiotherapy) Mental Health Data Prize Africa reposito	various	weak	Snippet	mentalhealthdataprizeafrica.aphrc.org/repository/paper/293
25	Using mobile phones to improve clinic attendance amongst an antiretroviral treatment cohort in rural Uganda: a cross-sectional and prospective study AIDS and Behavior (Kunutsor et al.), via	2010	moderate	Full	read.qxmd.com/read/20700644/using-mobile-phones-to-improve
26	Use of Electronic Health Records in sub-Saharan Africa: Progress and challenges Journal of Medicine in the Tropics 14(1)	2012	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4167769
27	Assessing the impact of a primary care electronic medical record system in three Kenyan rural health centers JAMIA, via IU ScholarWorks	2016	weak	Snippet	scholarworks.indianapolis.iu.edu/items/214a9c1d-f35c-4342-
28	Developing a point-of-care electronic medical record system for TB/HIV co-infected patients: experiences from Lighthouse Trust, Lilongwe, Malawi BMC Research Notes 9:146 (Tweya et al.)	2016	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4779573/
29	Effect of Text Message, Phone Call, and In-Person Appointment Reminders on Uptake of Repeat HIV Testing among Outpatients Screened for Acute HIV Infection in Kenya: A Ran PLOS ONE 11(4):e0153612 (Mugo et al.)	2016	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4831710/
30	SMS reminders and postnatal clinic attendance, Pumwani Referral Maternity Hospital (title not confirmed) Search snippet (AJOL)	2016	weak	Snippet	ajol.info/index.php/jieph/article/view/225726
31	Meta-analysis of Mobile Phone Reminders on HIV Patients' Retention to Care Journal of Mobile Technology in Medicine	2017	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC6124685

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32	Systemic Barriers to MomConnect's Capacity to Reach Registration Targets: A Process Evaluation MEASURE Evaluation, UNC Chapel Hill (TR-	2017	strong	Full	measureevaluation.cpc.unc.edu/resources/publications/tr-16
33	MomConnect reach and performance (BMJ Global Health supplement) BMJ Global Health 3(Suppl 2):e000583	2018	moderate	Snippet	gh.bmj.com/content/3/Suppl_2/e000583
34	Only five out of 696 hospitals and clinics got a pass mark in SA TimesLIVE	2018	weak	Snippet	timeslive.co.za/news/south-africa/2018-06-06-only-five-out
35	Effect of Phone Text Messages on Postnatal Clinic Attendance among Mothers Delivering at Mulago-Kawempe Hospital, A randomized controlled trial Makerere University (Kiwanuka P.K., thes	2019	moderate	Full	dspace.mak.ac.ug/items/7afa2c80-5c85-427e-bc53-6ad171a8367
36	Appointments: A More Effective Commitment Device for Health Behaviors arXiv 2110.06876	2021	weak	Snippet	arxiv.org/pdf/2110.06876
37	Implementation challenges and perception of care providers on Electronic Medical Records at St. Paul's and Ayder Hospitals, Ethiopia BMC Medical Informatics and Decision Mak	2021	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC8561912
38	Users' perception on factors contributing to electronic medical records systems use: a focus group discussion study in healthcare facilities setting in Kenya BMC Medical Informatics and Decision Mak	2021	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC8710176/
39	Healthcare problems persist at 'ideal clinics', studies show (Cele, Wits, Ideal Clinic rapid review) Spotlight NSP	2022	moderate	Full	spotlightnsp.co.za/2022/09/19/healthcare-problems-persist-
40	User Perceptions and Use of an Enhanced Electronic Health Record in Rwanda With and Without Clinical Alerts: Cross-sectional Survey JMIR Medical Informatics 10(5):e32305 (F	2022	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC9115652
41	Waiting time and its associated factors in patients presenting to outpatient departments at Public Hospitals of Jimma Zone, Southwest Ethiopia BMC Health Services Research (search sni	2022	moderate	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC8790858

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42	National Guideline on Management of Patient Waiting Time in Clinics, Community Health Centres and Outpatient Departments of Public Hospitals of South Africa National Department of Health, South Afr	2023	strong	Full	health.gov.za/wp-content/uploads/2024/05/Approved-national
43	Data quality evaluation of OpenMRS in Rwanda (Fraser et al.) JMIR Public Health and Surveillance 10:e	2024	moderate	Snippet	doaj.org/article/27d350dc08ee41aeae3f05c9fca62a41
44	KCMC outpatient waiting time preprint (medRxiv 10.1101/2024.02.14.24302723) medRxiv	2024	weak	Snippet	medrxiv.org/content/10.1101/2024.02.14.24302723.full.pdf
45	Outcome evaluation of technical strategies on reduction of patient waiting time in the outpatient department at Kilimanjaro Christian Medical Centre, Northern Tanzania BMC Health Services Research 24 (Mwanswi	2024	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC11234691/
46	Overall time spent by clients from entry to exit and associated factors in out-patient departments in public hospitals of Jimma Zone, southwest Ethiopia PLOS ONE 19(3):e0296630 (Walle et al.)	2024 (data 2018)	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10919670
47	Central Chronic Medicines Dispensing and Distribution programme abstracts and SENTINEL policy brief Population Medicine abstract; Boston Uni	2026	weak	Snippet	sites.bu.edu/ambit/files/2023/08/SENTINEL-1-SA-client-sati
48	IEA flags funding gaps, implementation flaws in SHA rollout; SHA system glitches force hospitals to demand cash payments Citizen Digital; Eastleigh Voice	2026	weak	Snippet	citizen.digital/news/iea-flags-funding-gaps-implementation
49	Outpatient waiting and appointment study of urban hospitals in Lusaka (title not confirmed) Search snippet (Medical Journal of Zambi	2026	unverifiable	Snippet	mjz.co.zm/index.php/mjz/article/download/862/654/5263
50	EMR implementation factors, Addis Ababa government hospitals (AAU thesis) Addis Ababa University ETD	unknown	weak	Snippet	etd.aau.edu.et/items/8f1297ce-8286-4912-9619-945a608b6909/
51	Queue analysis, St Paul's Hospital Millennium Medical College (AAU ETD repository) Addis Ababa University ETD	unknown	weak	Snippet	etd.aau.edu.et/bitstreams/ff6d8ba1-7012-4001-9a2d-fd811ba2
52	Languages of Cameroon (census-derived figures) and 2022 health communication study snippet Wikipedia / AJOL snippet	2005	weak	Snippet	en.wikipedia.org/wiki/Languages_of_Cameroon

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53	Efficiency of an intervention package for arterial hypertension comprising telemanagement in a Cameroonian rural setting: The TELEMED-CAM study Pan African Medical Journal	2013	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC3880813/
54	Tele-ECG controlled study and companion medico-economic analysis (Bediang et al.), via search listing IOS Press / Pan African Medical Journal	2016	unverifiable	Snippet	ebooks.iospress.nl/pdf/doi/10.3233/SHTI210244
55	Ideal Clinic waiting time evaluation, KwaZulu-Natal (SAMJ 108(4)) South African Medical Journal	2018	weak	Snippet	scielo.org.za/pdf/samj/v108n4/19.pdf
56	WHO guideline: recommendations on digital interventions for health system strengthening WHO (via NCBI Bookshelf listing)	2019	weak	Snippet	wkc.who.int/resources/publications/i/item/9789241550505
57	Barriers to Sustainable Telemedicine Implementation in Ethiopia: A Systematic Review PMC (snippet via web search)	2020	weak	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC8812291/
58	An Interactive Voice Response Software to Improve the Quality of Life of People Living With HIV in Uganda: Randomized Controlled Trial JMIR mHealth and uHealth (Byonanebye et	2021	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7906832/
59	Framework for implementing the Global strategy on digital health in the WHO African Region (AFR/RC71/10) WHO Regional Office for Africa	2021	unverifiable	Snippet	afro.who.int/sites/default/files/2021-11/AFR-RC71-10%20Fra
60	Mobile phone requirement for mHealth in Africa could exclude the most vulnerable (Okano et al., eLife) Newswise press release on eLife paper	2022	moderate	Full	newswise.com/articles/mobile-phone-requirement-for-mhealth
61	ReSAF situational analysis (digital health, Francophone Africa) Speak Up Africa	2022	weak	Snippet	speakupafrica.org/wp-content/uploads/2022/12/ReSAF-Analyse
62	The effectiveness and characteristics of mHealth interventions to increase adolescents' use of sexual and reproductive health services in sub-Saharan Africa: A syste PLoS ONE (Onukwugha et al.)	2022	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC8782484/
63	Two-way text message interventions and healthcare outcomes in Africa PLoS ONE (Odegard et al.)	2022	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC9009629/

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64	Understanding barriers of receiving short message service appointment reminders across African regions: a systematic review BMJ Health & Care Informatics (Demsa	2022	weak	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC9693653/
65	Africa CDC aims to be more tech-savvy with new digital health strategy Devex (search snippet)	2023	weak	Snippet	devex.com/news/africa-cdc-aims-to-be-more-tech-savvy-with-
66	An Unstructured Supplementary Service Data System to Verify HIV Self-Testing Among Nigerian Youths: Mixed Methods Analysis of Usability and Feasibility PMC (snippet via web search)	2023	weak	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC10562967/
67	Implementing teleconsultation in Low- and Middle-Income Countries (LMICs): a scoping review of barriers and facilitators Cochrane Global Evidence Summit 2024 abs	2024	weak	Snippet	abstracts.cochrane.org/2024-prague-global-evidence-summit/
68	Barriers and facilitators of provision of telemedicine in Nigeria: A systematic review PLOS Digital Health (Cole et al.)	2025	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12244542/
69	Editorial: Socioeconomic inequalities in digital health Frontiers in Digital Health	2025	weak	Full	frontiersin.org/journals/digital-health/articles/10.3389/f
70	Impact evaluation of a digital health platform empowering Kenyan women across the pregnancy-postpartum care continuum (Vatsa et al.) PLOS Medicine (preprint on medRxiv, snip	2025	moderate	Snippet	hks.harvard.edu/centers/cid/publications/impact-evaluation
71	Progress closing the mobile internet gender gap stalls in LMICs: GSMA Mobile Gender Gap Report 2025 GSMA	2025	strong	Full	gsma.com/newsroom/press-release/progress-closing-the-mobil
72	Telemedicine Adoption and Prospects in Sub-Sahara Africa: A Systematic Review with a Focus on South Africa, Kenya, and Nigeria Healthcare (Basel), Agbeyangi and Lukose	2025	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC11989057/
73	The State of Mobile Internet Connectivity Report 2025 GSMA	2025	moderate	Full	gsma.com/newsroom/?p=23943
74	World Health Assembly endorses extension of the global digital health strategy to 2027 WHO	2025	strong	Full	who.int/news/item/23-05-2025-world-health-assembly-endorse

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75	Mapping the use of interactive voice response (IVR) in healthcare in Africa: a scoping review protocol Mbarara University of Science and Techno	2026	weak	Snippet	ir.must.ac.ug/handle/123456789/4490
76	SkipTheQ: impact of a digital appointment system on efficiency Dialogues in Health via Maratto Africa I	2026	weak	Snippet	maratto.africa/research/skiptheq-impact-of-a-digital-appoi
77	Who Gets Left Behind? A Systematic Review of Marginalized Populations in Africa's Digital Health Ecosystem Population Medicine	2026	weak	Snippet	populationmedicine.eu/Who-Gets-Left-Behind-A-Systematic-Re
78	Cameroon to fully digitise payment in public hospitals as fraud cases rise ITWeb Africa	2022	moderate	Full	itweb.africa/article/cameroon-to-fully-digitise-payment-in
79	Cameroonian e-health platform Waspito secures US$2.7m ITWeb Africa	2022	moderate	Full	itweb.africa/content/RgeVDMPY6p1qKJN3
80	Offres Grand Public 1ere Partie (TC 2023) Agence de Regulation des Telecommunicati	2023	moderate	Snippet	art.cm/sites/default/files/documents/Offres%20Grand%20Publ
81	Willingness to use EMR among health professionals, Amhara private hospitals PLOS ONE via PMC	2023	moderate	Snippet	ncbi.nlm.nih.gov/pmc/articles/PMC10150984/
82	Cost-effectiveness of two-way texting for ART retention (medRxiv preprint) medRxiv	2024	unverifiable	Snippet	medrxiv.org/content/10.1101/2024.04.17.24305960v1.full.pdf
83	Government awards Safaricom consortium KSh 104 billion digitization tender Health Business Kenya	2024	moderate	Full	healthbusiness.co.ke/8219/government-awards-safaricom-cons
84	State of Broadband 2025 ITU	2024	moderate	Snippet	developmentaid.org/api/frontend/cms/file/2025/06/2500663E.
85	Appointment SMS reminders and no-shows Kolonell (vendor blog)	2026	weak	Full	kolonell.com/en/blog/appointment-sms-reminders-no-show-kam
86	Business messaging on MTN Cameroon Messaggio	2026	weak	Full	messaggio.com/operators/mtn-cameroon/
87	Cameroon SMS pricing FAQ Sent.dm	2026	weak	Full	sent.dm/resources/cameroon-sms-pricing
88	Clinic management software Abidjan cost Kolonell (vendor blog)	2026	weak	Full	kolonell.com/en/blog/clinic-management-software-abidjan-co

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89	EMR DHIS2 integration survey DHIS2 developers blog	2026	weak	Snippet	developers.dhis2.org/blog/2026/06/emr-dhis2-integration-su
90	HGY 26 JUIN MP NEW DAO INFO ARMP Cameroon (pridesoft)	2026	unverifiable	Snippet	pridesoft.armp.cm/_lib/file/doc/0903/2026/07/01/HGY/HGY%20
91	Hospital management software Africa Streamline Health	2026	weak	Snippet	streamlinehealth.org/hospital-management-software-africa/
92	Hospital management software French-speaking Douala Kolonell (vendor blog)	2026	weak	Full	kolonell.com/en/blog/hospital-management-software-french-s
93	LinkHMS Capterra South Africa	2026	weak	Snippet	capterra.co.za/software/1072624/LinkHMS
94	NDoH04 2025/2026 Invitation to Bid EMR implementation National Department of Health South Afri	2026	unverifiable	Snippet	health.gov.za/wp-content/uploads/2026/03/NDoH04-20252026-F
95	Private hospital management system cost Kolonell (vendor blog)	2026	weak	Full	kolonell.com/en/blog/private-hospital-management-system-co
96	Projet Catalogue Interco Camtel BUM Camtel	2026	unverifiable	Snippet	camtel.cm/wp-content/uploads/2024/01/Projet-CatalogueInter
97	SMS Pricing Cameroon Plivo	2026	moderate	Full	plivo.com/sms/pricing/cm/
98	SMS Pricing Cameroon Twilio	2026	moderate	Full	twilio.com/en-us/sms/pricing/cm
99	SMS gateway for Cameroon BudgetSMS	2026	moderate	Full	budgetsms.net/sms-gateway-pricing/cm/cameroon
100	Voice pricing Cameroon Twilio	2026	moderate	Full	twilio.com/en-us/voice/pricing/cm
101	Celcom Africa bulk SMS Cameroon / QuestionPro SMS pricing table Celcom Africa; QuestionPro	unknown	weak	Snippet	celcomafrica.com/bulk-sms-cameroon
102	Cost analysis of electronic integrated disease surveillance and response, Western Area Rural CDC Stacks	unknown	moderate	Snippet	stacks.cdc.gov/view/cdc/114245/cdc_114245_DS1.pdf
103	EMRs save lives, why are we still debating if we fund them ICTworks (opinion)	unknown	weak	Full	ictworks.org/emrs-save-lives-why-are-we-still-debating-if-

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104	Study: South Africans ready for e-medical records Computerwoche	unknown	weak	Snippet	computerwoche.de/a/study-south-africans-ready-for-e-medica
105	Telephonie mobile: Camtel pratique les prix les plus bas Ecomatin	unknown	weak	Snippet	ecomatin.net/telephonie-mobile-camtel-pratique-les-prix-le
106	Mobile phone messages improve adherence and HIV control in Kenyan trial aidsmap	2010	weak	Snippet	aidsmap.com/news/nov-2010/mobile-phone-messages-improve-ad
107	Cochrane CD007458 information page Cochrane Library	2013	strong	Snippet	cochranelibrary.com/cdsr/doi/10.1002/14651858.CD007458.pub
108	How SMS is helping to save children's lives in Cameroon GSMA mPowering Youth	2013	unverifiable	Snippet	gsma.com/mpoweryouth/?p=51595
109	Mobile Tech Issue: Tracking Disease, One Text at a Time TIME Healthland	2013	weak	Snippet	healthland.time.com/?p=66528
110	Mobile phone messaging reminders for attendance at healthcare appointments Cochrane Database of Systematic Reviews	2013	strong	Full	doi.org/10.1002/14651858.CD007458.pub3
111	Pilot-itis: What's the cure? GSMA mHealth programme	2013	moderate	Snippet	gsma.com/solutions-and-impact/connectivity-for-good/mobile
112	SUPPORT Summary of Gurol-Urganci systematic review Epistemonikos SUPPORT summaries	2013	weak	Snippet	supportsummaries.epistemonikos.org/assets/Uploads/Gurol-Ur
113	mHealth for maternal health: mHealth pilotitis, time for recovery or relapse Harvard T.H. Chan School of Public Healt	2013	unverifiable	Snippet	hsph.harvard.edu/maternal-health-task-force/news/mhealth-f
114	A mixed methods approach to prioritizing components of Uganda's eHealth environment Pan African Medical Journal 22:198 (Muko	2015	strong	Full	panafrican-med-journal.com/content/article/22/198/full/
115	BMC Medical Informatics and Decision Making article s12911-017-0413-9 (cost of mobile reminders) BMC Medical Informatics and Decision Mak	2017	weak	Snippet	bmcmedinformdecismak.biomedcentral.com/track/pdf/10.1186/s
116	What is the economic evidence for mHealth? A systematic review of economic evaluations of mHealth solutions PLoS ONE (Iribarren et al.)	2017	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC5289471/

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117	Best practices in scaling digital health in low and middle income countries Globalization and Health (Labrique et al	2018	moderate	Snippet	doi.org/10.1186/s12992-018-0424-z
118	Data sharing via SMS strengthens Uganda's health system (mTRAC case study) UN SDSN TReNDS	2018	weak	Snippet	unsdsn.org/resources/data-sharing-via-sms-strengthens-ugan
119	Mobile health messaging service and helpdesk for South African mothers (MomConnect): history, successes and challenges BMJ Global Health (Barron et al.), doi 1	2018	strong	Snippet	doi.org/10.1136/bmjgh-2017-000559
120	MomConnect cost-effectiveness modelling (as summarised in search result) BMJ Global Health supplement (search sni	2018	weak	Snippet	gh.bmj.com/content/3/Suppl_2/e000562
121	The impact of an mHealth monitoring system on health care utilization by mothers and children: an evaluation using routine health information in Rwanda Health Policy and Planning 33(8):920–927	2018	strong	Snippet	doi.org/10.1093/heapol/czy066
122	WHO guideline: recommendations on digital interventions for health system strengthening World Health Organization	2019	moderate	Snippet	ncbi.nlm.nih.gov/books/NBK541905/
123	JMIR mHealth and uHealth 2022;10(6):e18188 (pilotitis in South Africa) JMIR mHealth and uHealth	2022	weak	Snippet	mhealth.jmir.org/2022/6/e18188/PDF
124	Community participation for RMNCH: insights from design and implementation of BornFyne-PNMS in Cameroon Frontiers in Digital Health 5:1218641 (O	2023	strong	Full	frontiersin.org/articles/10.3389/fdgth.2023.1218641
125	Short Message Service (SMS) Reminders to Increase Infant Vaccination GiveWell	2023	weak	Snippet	givewell.org/node/2862
126	Cost-effectiveness analysis of two-way texting (2wT) intervention to improve ART retention among newly-initiated ART clients in Malawi medRxiv preprint (Kiruthu-Kamamia et al.	2024	moderate	Snippet	medrxiv.org/content/10.1101/2024.04.17.24305960v1
127	Costs and cost-effectiveness of interventions to improve HIV treatment adherence in Cape Town, South Africa Boston University Open Access (journal n	2024	weak	Snippet	open.bu.edu/items/42d836b0-bfa1-49d9-9eea-2b8dcb9e022d

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128	A Tailored mHealth Intervention for Improving Antenatal Care Seeking Among Pregnant Adolescent Girls and Young Women in South Africa: Pilot RCT JMIR mHealth and uHealth (Sewpaul et al.	2025	moderate	Snippet	doi.org/10.2196/59144
129	AfricaHF Transition Readiness (draft) Africa Health Financing initiative (draf	2025	weak	Snippet	ahfia.belvadigital.com/wp-content/uploads/2025/11/AfricaHF
130	Beyond seed funding: why Nigerian digital health startups struggle to grow Health Economics Review 16 (Elechi et al	2025	moderate	Snippet	doi.org/10.1186/s13561-025-00708-6
131	Gavi and Global Fund replenishments: donors must demand real reform Center for Global Development	2025	weak	Snippet	cgdev.org/blog/gavi-and-global-fund-replenishments-donors-
132	Helium Credit Is Financing Africa's Private Clinics, One Loan At a Time; Helium Health secures 30M allAfrica / TechCabal / Nigeria Health W	2025	weak	Snippet	allafrica.com/stories/202509300039.html
133	Impact of SMS reminders or peer home visits on ART adherence and viral load suppression among HIV-infected adolescents in Cameroon: RCT AIDS Research and Therapy 22:49 (Ketchaj	2025	moderate	Snippet	doi.org/10.1186/s12981-025-00746-4
134	SMS reminder RCT on childhood immunization in Gurage Zone, Ethiopia BMC Public Health (Gebreeyesus et al.),	2025	moderate	Snippet	doi.org/10.1186/s12889-025-25680-y
135	Barriers and facilitators to implementation and scale-up of Akoma Pa mHealth integrated care programme BMC Health Services Research 26:304 (Str	2026	moderate	Snippet	doi.org/10.1186/s12913-026-14175-0
136	Cost and Cost-Effectiveness of mHealth Interventions to Promote Uptake of ANC in LMICs: A Scoping Review Cureus (Atuhumuza et al.), doi 10.7759/c	2026	moderate	Snippet	doi.org/10.7759/cureus.110807
137	Coverage, pricing, and reimbursement of telemedicine: Akoma Pa implementers, Ghana BMC Health Services Research 26 (Bonney	2026	moderate	Snippet	doi.org/10.1186/s12913-026-15091-z
138	Economic evaluation of digital health interventions to improve QALYs in adolescents living with HIV in Ethiopia Frontiers in Public Health (Tunje et al.	2026	strong	Full	ncbi.nlm.nih.gov/pmc/articles/PMC12876147/

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139	Effect of Supervision or SMS on Detection Rate of Diseases Under Surveillance in the Littoral Region of Cameroon Research and Reports in Tropical Medicin	2026	moderate	Snippet	doi.org/10.2147/rrtm.s622004
140	Effectiveness of Digital Health Interventions for Improving ART Outcomes in People With HIV JMIR (Liu et al.), doi 10.2196/81019	2026	moderate	Snippet	doi.org/10.2196/81019
141	Synthesis of Europe PMC searches (cost AND SMS AND attendance AND Africa) Europe PMC search, searched 2026-11-12	2026	moderate	Snippet	ebi.ac.uk/europepmc/webservices/rest/search?query=%22text%
142	Text message reminders and attendance during the sociopolitical crisis in Haiti JMIR Formative Research (Presume et al.)	2026	weak	Snippet	doi.org/10.2196/77010
143	WSIS Prizes 2026 project details, MomConnect ITU WSIS Stocktaking	2026	weak	Snippet	itu.int/net4/wsis/stocktaking/Prizes/2026/Details/17720121
144	Zuri Health expands mobile clinic fleet across high-traffic Nairobi TechCabal	2026	weak	Snippet	techcabal.com/2026/04/14/zuri-health-expands-nairobi-fleet
145	Trends and determinants of unfavourable outcomes in paediatric tuberculosis: insights from a 20-year cohort in Cameroon BMJ Open Respiratory Research, Poka-Maya	2001–2020	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12406847/
146	Factors associated with non-adherence to scheduled medical follow-up appointments among Cameroonian children requiring HIV care Infectious Diseases of Poverty, Bigna et	2013	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4322435/
147	Non-adherence to antihypertensive pharmacotherapy in Buea, Cameroon: a cross-sectional community-based study BMC Cardiovascular Disorders, 2018	2013–2014	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC6056997/
148	Retention on antiretroviral therapy at 12 months in Cameroon (nationwide) BMC Infectious Diseases, 2016	2013–2014	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC5111226/
149	SMS reminders to improve adherence and cure of tuberculosis patients in Cameroon (TB-SMS Cameroon): a randomised controlled trial BMC Public Health, 2018	2013–2014	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC5932834/

#	SOURCE	YEAR	CONFIDENCE	READ	URL
150	Clients' satisfaction with HIV treatment services in Bamenda, Cameroon: a cross-sectional study BMC Health Services Research, 2016	2014	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4950718/
151	Health Services Utilisation before and during an Armed Conflict; Experiences from the Southwest Region of Cameroon The Open Public Health Journal, 2020	2016–2018	unverifiable	Snippet	benthamopen.com/ABSTRACT/TOPHJ-13-547
152	Clinical characteristics and outcomes of tuberculosis in Douala, Cameroon: a 7-year retrospective cohort study search result summary	2017	weak	Snippet	pubmed.ncbi.nlm.nih.gov/28000583/
153	Why do pregnant women present late for their first antenatal care consultation in Cameroon? Maternal Health, Neonatology and Perinat	2017	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC5727881/
154	Cameroon doctors overwhelmed with patients Voice of America	2018	weak	Full	voanews.com/a/cameroon-doctors-overwhelmed-with-patients/4
155	EPI immunization coverage, timeliness and dropout rate among children in a West Cameroon health district: a cross sectional study BMC Public Health, 2020	2018	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7020570/
156	Waiting Time and Patient Satisfaction in a Subspecialty Eye Hospital Using a Mobile Data Collection Kit: Pre-Post Quality Improvement Intervention JMIRx Med, Mbwogge et al.	2018	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10414230/
157	Evaluating turnaround time to improve clients' satisfaction in the tuberculosis reference laboratory in Douala PLoS One, Mbuh et al., 2025	2022	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12165355/
158	Type 2 diabetes glycaemic control and follow-up, Bafoussam Regional Hospital (via search summary) BMC Endocrine Disorders (article id s129	2022	unverifiable	Snippet	bmcendocrdisord.biomedcentral.com/track/pdf/10.1186/s12902
159	Assessing health service satisfaction among beneficiaries in Cameroon: a cross-sectional study to inform healthcare improvement BMC Health Services Research, Ndie et al	2022–2023	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12911053/

#	SOURCE	YEAR	CONFIDENCE	READ	URL
160	Determinants of patient satisfaction with healthcare services among insured and uninsured patients at a public hospital in Cameroon: modified PSQ-18 Journal of Public Health Research, Cynth	2022–2023	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12181729/
161	Impact of telephone reminders on posttreatment follow-up among women treated for cervical precancers in Cameroon: a randomized controlled trial Gynecologic Oncology Reports, Nkfusai et	2022–2023	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12041788/
162	The loss to follow-up of participants excluded from the NAMSAL trial: a retrospective cohort study Scientific Reports, Goufack Kenfack et a	2022–2023	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12647855/
163	2023 CQUIN Cameroon country poster (MOH updates) ICAP at Columbia / CQUIN	2023	unverifiable	Snippet	cquin.icap.columbia.edu/wp-content/uploads/2023/11/2023_CQ
164	Improving Childhood Immunization Service Delivery in Cameroon: A Synthesis of Caregiver Experiences and Recommendations Vaccines (Basel), Saidu et al.	2023	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC11728739/
165	Investigating predictors and barriers to vaccine uptake in children at Konye Health District, South West region Cameroon BMC Public Health, Manka et al., 2026	2023	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC13628756/
166	Retention in care for type 2 diabetes management in Sub-Saharan Africa: A systematic review Tropical Medicine and International Heal	2023	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10947597/
167	The Effect of Implementing Advanced Strategies to Improve Immunization Coverage in the Elogbele Health Area Cureus, Ndjip Ndjock et al., 2025	2023	weak	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12227138/
168	A 360 Degree Qualitative Assessment of mHealth Follow-up in Cameroon Journal of Surgical Research, Yost et al	2023–2024	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC13122765/
169	Knowledge, health-seeking behavior, and predictors of treatment adherence among tuberculosis patients in the North-West Region of Cameroon Pan African Medical Journal, 2026	2024–2025	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC13477182/

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170	Pregnant women's perspectives on antenatal care utilization in Cameroon: a cross-sectional mixed-methods study Frontiers in Global Women's Health,	2024–2025	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC13124922/
171	BBH reports over 1,000 NCD patients enrolled as hospital steps up fight in conflict Hilltop Voices	2025	weak	Full	hilltopvoices.com/2025/06/bbh-reports-over-1000-ncd-patien
172	Search null result across Europe PMC and web search n/a	2026	moderate	Snippet	europepmc.org/
173	Thesis on waiting time and satisfaction at Yaounde Gynaeco-Obstetric and Paediatric Hospital (title not seen) DICAMES repository (CAMES)	2026	unverifiable	Snippet	dicames.online/jspui/handle/20.500.12177/13613
174	Adherence to antihypertensive treatment studies (Garoua, Bamenda, Yaounde), via search summary memoireonline and hsd-fmsb.org	various	unverifiable	Snippet	hsd-fmsb.org/index.php/hsd/article/download/904/pdf_475/26
175	Designing and implementing an electronic health record system in primary care practice in sub-Saharan Africa: a case study from Cameroon Informatics in Primary Care (Kamadjeu et	2005	weak	Snippet	doi.org/10.14236/jhi.v13i3.595
176	Referral of patients with kidney impairment for specialist care in a developing country of sub-Saharan Africa Renal Failure (Halle et al.)	2009	weak	Snippet	doi.org/10.1080/08860220902882014
177	The interface between the national tuberculosis control programme and district hospitals in Cameroon: missed opportunities for strengthening the local health system BMC Public Health (Keugoung et al.)	2013	moderate	Snippet	doi.org/10.1186/1471-2458-13-265
178	Challenges to Implementing a National Health Information System in Cameroon: Perspectives of Stakeholders Journal of Public Health in Africa (Ngwa	2014	moderate	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC5345461/
179	Adequacy of UTAUT in clinician adoption of health information systems in developing countries: the case of Cameroon International Journal of Medical Informa	2018	moderate	Snippet	doi.org/10.1016/j.ijmedinf.2017.10.016

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180	Viral load testing and the use of test results for clinical decision making for HIV treatment in Cameroon PLoS ONE (Awungafac et al.)	2018	moderate	Snippet	doi.org/10.1371/journal.pone.0198686
181	The mother's carnet de sante (health booklet) in Cameroon: a tool for preventing mother-to-child transmission of HIV? Anthropology and Medicine (Tantchou, Tij	2019	weak	Snippet	oskar-bordeaux.fr:443/handle/20.500.12278/133241
182	Evaluation of corrective actions of feedback from clinicians on Clinical Laboratory Services at Bamenda Regional Hospital Laboratory, Cameroon African Journal of Laboratory Medicine (	2020	moderate	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC7136689/
183	Factors associated with the performance of routine health information system in Yaounde-Cameroon: a cross-sectional survey BMC Medical Informatics and Decision Mak	2020	moderate	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC7745475/
184	Routine health information system in the health facilities in Yaounde-Cameroon: assessing the gaps for strengthening BMC Medical Informatics and Decision Mak	2020	moderate	Snippet	doi.org/10.1186/s12911-020-01351-3
185	Patient-Pathway Analysis of Tuberculosis Services in Cameroon Tropical Medicine and Infectious Disease	2021	strong	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC8544654/
186	Govt to set up e-patient record system in Cameroon StopBlaBlaCam (news)	2022	weak	Full	stopblablacam.com/society/1511-9677-govt-to-set-up-e-patie
187	Hopitaux publics: apres la numerisation des paiements, place a la digitalisation des dossiers medicaux Ecomatin (news)	2022	weak	Snippet	ecomatin.net/hopitaux-publics-apres-la-numerisation-des-pa
188	Interventions to improve obstetric emergency referral decision making, communication and feedback between health facilities in sub-Saharan Africa Tropical Medicine and International Heal	2022	moderate	Snippet	doi.org/10.1111/tmi.13747
189	Microbiology laboratories involved in disease and antimicrobial resistance surveillance: strengths and challenges in ECCAS African Journal of Laboratory Medicine	2022	moderate	Snippet	doi.org/10.4102/ajlm.v11i1.1570

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190	Assessment of immunization data management practices in Cameroon: unveiling potential barriers to immunization data quality BMC Health Services Research (Saidu et a	2023	strong	Snippet	link.springer.com/10.1186/s12913-023-09965-9
191	Cameroon to introduce electronic medical records ITWeb Africa	2023	moderate	Full	itweb.africa/content/DZQ58MV8wXpvzXy2
192	Implementing Clinical Information Systems in Sub-Saharan Africa: Report and Lessons Learned From the MatLook Project in Cameroon JMIR Medical Informatics (Bediang G)	2023	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10620639/
193	Usefulness of disease surveillance data in enhanced early warning of the cholera outbreak in Southwest Cameroon 2018 Conflict and Health	2023	moderate	Snippet	doi.org/10.1186/s13031-023-00504-1
194	Developing the BornFyne prenatal management system version 2.0 Oxford Open Digital Health (Nkangu et al	2024	weak	Snippet	doi.org/10.1093/oodh/oqae012
195	Assessing the landscape of digital health implementation for data services in Cameroon Digital Health (SAGE), Nsai et al.	2025	moderate	Snippet	doi.org/10.1177/20552076251395551
196	CQUIN M&E Systems Vulnerability Assessment - Cameroon Pilot Findings (Session 9b) CQUIN / ICAP Columbia, presented by Nati	2025	strong	Full	cquin.icap.columbia.edu/wp-content/uploads/2025/11/Session
197	Determinants of patient satisfaction with healthcare services among insured and uninsured patients at a public hospital in Cameroon Journal of Public Health Research (Cynth	2025	moderate	Snippet	doi.org/10.1177/22799036251345936
198	Health Modernization: Gado-Badzere's Health Workers Go Digital UNICEF Cameroon	2025	weak	Full	unicef.org/cameroon/stories/health-modernization-gado-badz
199	Referral and outcome of out born neonates to the neonatology unit of the Bamenda Regional Hospital, Cameroon Annals of Community Medicine and Primary	2025	strong	Snippet	meddocsonline.org/annals-of-community-medicine-and-primary
200	Assessing the alignment of the Cameroon health information system data sources with international standards: a mixed sequential study Research Square preprint (Ombogo et al.)	2026	weak	Snippet	doi.org/10.21203/rs.3.rs-10135250/v1

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201	Assessing the functionality of the routine health information system in health facilities of Yaounde, Cameroon: a cross-sectional study Research Square preprint (Dang et al.)	2026	weak	Snippet	doi.org/10.21203/rs.3.rs-10582695/v1
202	Barriers to the functionality and responsiveness of the malaria surveillance and data use system in Cameroon Malaria Journal (Nkfusai et al.)	2026	moderate	Snippet	doi.org/10.1186/s12936-026-06091-9
203	Determinants of the willingness to use an electronic medical record at a category one hospital in Cameroon Research Square preprint (Essono et al.)	2026	weak	Snippet	doi.org/10.21203/rs.3.rs-10279549/v1
204	Restructuring national health information system software to improve malaria data quality Research Square preprint	2026	moderate	Snippet	researchsquare.com/article/rs-11014186/v1
205	Timeliness of acute stroke care and predictors of delays in three referral hospitals in Douala, Cameroon Research Square preprint	2026	weak	Snippet	doi.org/10.21203/rs.3.rs-10844057/v1
206	Cameroon public health sector: shortage and inequalities in geographic distribution of health personnel International Journal for Equity in Heal	2011	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4440287/
207	Ratios and determinants of maternal mortality: a comparison of geographic differences in the northern and southern regions of Cameroon BMC Pregnancy and Childbirth 20:194 (Meh	2011	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7110763
208	The effect of the ongoing civil strife on key immunisation outcomes in the North West and South West regions of Cameroon Conflict and Health 15:8 (Saidu et al.,	2016–2019	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7877013
209	First lawyers and teachers, now doctors are striking in Cameroon International Business Times UK	2017	weak	Full	ibtimes.co.uk/first-lawyers-teachers-now-doctors-are-strik
210	Health Services Utilisation before and during an Armed Conflict; Experiences from the Southwest Region of Cameroon The Open Public Health Journal 13 (Haddi	2017–2018	moderate	Full	openpublichealthjournal.com/VOLUME/13/PAGE/547/ABSTRACT/
211	Distance to health facility table, multi-country DHS analysis (BMC Health Services Research) BMC Health Services Research (article 10	2018	weak	Snippet	bmchealthservres.biomedcentral.com/articles/10.1186/s12913

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212	Predictors of skilled birth attendance among married women in Cameroon: further analysis of 2018 Cameroon Demographic and Health Survey Reproductive Health 18:70 (Yaya et al.,	2018	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7993505/
213	Implementing updated health accounts to reduce financial hardship in Cameroon's healthcare system World Health Organization (country story	2019	strong	Full	who.int/about/accountability/results/who-results-report-20
214	Violence in North-West and South-West Cameroon: five things you need to know Medecins Sans Frontieres (MSF Kenya mirr	2019	moderate	Full	msf.or.ke/en/magazine/violence-north-west-and-south-west-c
215	Cameroon: Civilians Killed in Anglophone Regions Human Rights Watch (news release 27 July	2020	moderate	Full	hrw.org/news/2020/07/27/cameroon-civilians-killed-anglopho
216	Impact of periodic intensification of routine immunization within an armed conflict setting and COVID-19 outbreak in Cameroon in 2020 Conflict and Health 16:29 (Njoh et al.,	2020	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC9161648
217	Mixed effects analysis of factors associated with barriers to accessing healthcare among women in sub-Saharan Africa: Insights from demographic and health surveys PLoS One 15(11):e0241409 (Seidu, 2020)	2020	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7652334/
218	Cameroon: thousands of people continue to be denied urgent and life-saving health care Medecins Sans Frontieres (MSF Kenya mirr	2020–2021	moderate	Full	msf.or.ke/en/magazine/cameroon-thousands-people-continue-b
219	Factors associated with full vaccination and zero vaccine dose in children aged 12–59 months in 6 health districts of Cameroon BMC Public Health 23:1693 (Yakum et al.,	2021	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10474728/
220	Structural barriers to maternity care in Cameroon: a qualitative study Reproductive Health 21:108 (Harsono et a	2021	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC11264682/
221	Violence continues in Cameroon, access to healthcare remains seriously limited Medecins Sans Frontieres (MSF)	2021	moderate	Full	msf.org/violence-continues-cameroon-access-healthcare-rema
222	Conflict, healthcare and professional perseverance: A qualitative study in a remote hospital in an Anglophone Region of Cameroon PLOS Global Public Health 2(11):e0001145	2022	weak	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10021219/

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223	Search for geospatial accessibility to health facilities in Cameroon (null result); Kenya benchmark in Frontiers in Public Health 2022 Frontiers in Public Health (Kenya benchm	2022	unverifiable	Snippet	frontiersin.org/journals/public-health/articles/10.3389/fp
224	Cameroon: health personnel strike ongoing nationwide as of June 2 (update 1) Crisis24 (GardaWorld)	2023	weak	Snippet	crisis24.com/alerts/2023/06/cameroon-health-personnel-stri
225	Household costs incurred when seeking and receiving paediatric tuberculosis services: a survey in Cameroon and Kenya Journal of Global Health Reports (White	2023	weak	Snippet	eprints.whiterose.ac.uk/id/eprint/233069/1/88168-household
226	Performance-based financing, health sectors, and health seeking behavior of women attending antenatal care and skilled birth delivery: evidence from Cameroon Journal of Health Inequalities 9(1):89–1	2023	moderate	Full	termedia.pl/Performance-based-financing-health-sectors-and
227	Temporary workers in Cameroon's public hospitals Equal Times, 13 November 2023	2023	moderate	Full	equaltimes.org/temporary-workers-in-cameroon-s
228	Cameroon or Canada? Poorly paid doctors and nurses are choosing to leave Associated Press via Medical Xpress, 11	2024	moderate	Full	medicalxpress.com/news/2024-06-cameroon-canada-poorly-paid
229	2.3 Cameroon Road Network (Logistics Capacity Assessment) and Routes degradees: Le Cameroun parmi les 10 pires reseaux routiers du continent Logistics Cluster DLCA; AllAfrica (Frenc	2025	unverifiable	Snippet	dlca.logcluster.org/print-preview-current-section/1948
230	Cameroon: North-West and South-West regions overview of humanitarian access, January to December 2025 and related situation reports UN OCHA via ReliefWeb	2025	unverifiable	Snippet	reliefweb.int/report/cameroon/cameroon-north-west-and-sout
231	Care in the Crosshairs: Violence Against Health Care in Conflict, Cameroon 2025 Safeguarding Health in Conflict Coalitio	2025	strong	Full	insecurityinsight.org/wp-content/uploads/2026/04/2025-SHCC
232	Debt, detention and the universal healthcare gap: lessons from Fundong, Cameroon (summary of 'Trapped by debt', Frontiers in Public Health 2025) Human Sciences Research Council (HSRC)	2025	weak	Full	hsrc.ac.za/news/review/debt-detention-and-the-universal-he
233	Zero Dose Children and Vaccine Gaps in Cameroon Think Global Health (Council on Foreign	2025	weak	Full	thinkglobalhealth.org/article/Zero-Dose-Children-and-Vacci

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234	Cameroun fintech market sheet Team France Export (snippet only)		unverifiable	Snippet	teamfrance-export.fr/fiche-marche/tech-et-services/fintech
235	INS ICT access and usage survey presentation (UNCTAD-hosted) INS via UNCTAD (snippet only)		unverifiable	Snippet	unctad.org/node/43607
236	Ecosystem Accelerator Compass: GiftedMom case study GSMA Mobile for Development	2018	moderate	Full	gsma.com/mobilefordevelopment/wp-content/uploads/2019/08/G
237	Cameroon's start-ups urged to leverage national payment gateway ITWeb Africa	2021	moderate	Full	itweb.africa/article/cameroons-start-ups-urged-to-leverage
238	Cameroon SIM card registration news (ITWeb Africa, Business in Cameroon) ITWeb Africa and others (snippet only)	2022	unverifiable	Snippet	itweb.africa/article/cameroonian-telcos-in-hot-water-over-
239	ECAM5 SDG indicators page (English) Institut National de la Statistique (INS	2022	strong	Full	ins-cameroun.cm/wp-content/uploads/2025/07/Page-ODD-ECAM5-
240	Cameroon's telecom sector grows 18% in 2024 TechAfrica News (reporting ART 2024 Obse	2024	moderate	Full	techafricanews.com/2025/12/09/cameroons-telecom-sector-gro
241	Data Protection in Cameroon DLA Piper Data Protection Laws of the Wo	2024	moderate	Full	dlapiperdataprotection.com/index.html?c=CM&t=law
242	Fintech and payments regulation in Cameroon (law firm guides) Global Law Experts and similar (snippet	2024	unverifiable	Snippet	globallawexperts.com/fintech-consumer-protection-cameroon/
243	World Development Indicators IT.NET.USER.ZS and IT.CEL.SETS.P2, Cameroon World Bank (API)	2024	strong	Full	api.worldbank.org/v2/country/CMR/indicator/IT.NET.USER.ZS?
244	Africa's digital divide: insights from the Global Network Excellence Index Opensignal (snippet only)	2025	unverifiable	Snippet	insights.opensignal.com/2025/09/africas-digital-divide-ins
245	Cameroon SMS regulations (vendor guides) d7networks, sent.dm, Messente, Vonage (s	2025	unverifiable	Snippet	d7networks.com/sms/cameroon/sms-regulations/
246	Cameroon data protection law compliance LEX Africa	2025	weak	Full	lexafrica.com/2025/10/cameroon-data-protection-law-complia
247	Cameroun: l'Etat a preleve 43 milliards FCFA sur les transferts d'argent en 2025 Ecomatin (snippet only)	2025	unverifiable	Snippet	ecomatin.net/cameroun-letat-a-preleve-43-milliards-fcfa-su

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248	Detailed Report on Cameroon's Evolving Data Privacy Landscape Afriwise	2025	weak	Full	afriwise.com/resources/cameroons-evolving-data-privacy-lan
249	Digital 2026: Cameroon DataReportal (Kepios)	2025	moderate	Full	datareportal.com/reports/digital-2026-cameroon
250	Global Findex 2025 microdata catalog entry 7880 World Bank Microdata Library (snippet on	2025	unverifiable	Snippet	microdata.worldbank.org/catalog/7880
251	IMF Working Paper WP/25/255 (mobile money tax in Cameroon) International Monetary Fund	2025	moderate	Snippet	imf.org/-/media/files/publications/wp/2025/english/wpiea20
252	Internet disruptions reported in Cameroon as protests erupt over contested election Cameroon Online (citing NetBlocks)	2025	moderate	Full	cameroononline.org/internet-disruptions-reported-in-camero
253	Search results on Cameroon smartphone and USSD GSMA and Nkafu (snippet only)	2025	unverifiable	Snippet	nkafu.org/?p=115473
254	Updated: every African country's national digital health strategy in 2026 ICTworks	2025	moderate	Full	ictworks.org/updated-every-african-countrys-national-digit
255	CAMEROUN: La loi sur la protection des donnees a caractere personnel entre en vigueur ce 23 juin 2026 Droit Medias Finance	2026	weak	Full	droitmediasfinance.com/index.php/actualites/droit-tech-fin
256	Cameroon SMS Guidelines Twilio	2026	moderate	Full	twilio.com/en-us/guidelines/cm/sms
257	Cameroun: l'Etat injecte 29,7 milliards de FCFA dans le Plan strategique national de sante numerique 2026–2030 Digital Business Africa	2026	moderate	Full	digitalbusiness.africa/cameroun-letat-injecte-297-milliard
258	National Digital Health Strategic Plan 2026–2030 (English) MINSANTE, Republic of Cameroon	2026	strong	Full	minsante.cm/site/sites/default/files/NDHSP%202026-2030%20E
259	Search results: Cameroon Law 2024/017 data protection health data Multiple law and compliance sites (snipp	2026	unverifiable	Snippet	consentstack.io/regulations/cm-law2010012
260	The Mobile Economy Africa 2026 GSMA	2026	unverifiable	Snippet	gsma.com/solutions-and-impact/connectivity-for-good/mobile

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261	Au Cameroun, Idocta dematerialise les services medicaux CIO Mag	2019	moderate	Full	cio-mag.com/?p=19441
262	Cameroun: numerisation des carnets (Hospisoft) Osiris / CIO Mag	2019	moderate	Full	osiris.sn/Cameroun-numerisation-des-carnets.html
263	Au Cameroun et au Nigeria, Healthlane democratise l'acces aux soins de sante We Are Tech Africa	2021	moderate	Full	wearetech.africa/fr/fils/solutions/au-cameroun-et-au-niger
264	Health information systems in developing countries: case of African countries BMC Medical Informatics and Decision Mak	2021	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC8336100
265	Avec OuiCare, Emmanuel Assom introduit au Cameroun le livret medical digitalise We Are Tech Africa	2022	moderate	Full	wearetech.africa/home-fr-fr/fils/tech-stars/avec-ouicare-e
266	Healthlane owed salaries, layoffs Techpoint Africa	2022	weak	Full	techpoint.africa/2022/11/18/healthlane-owed-salaries-layof
267	Philippe Ohandja Ayina rapproche la medecine des Camerounais We Are Tech Africa; Cameroon Tribune (sn	2022	weak	Full	wearetech.africa/fr/fils/tech-stars/philippe-ohandja-ayina
268	Cameroonian healthtech raises additional seed Techpoint Africa	2023	moderate	Full	techpoint.africa/2023/11/22/cameroonian-healthtech-raises-
269	Ekose Holdings SARL VC4A	2023	unverifiable	Full	vc4a.com/ventures/ekose-holdings-sarl/
270	Healthlane was meant to make hospitals obsolete. Now the startup is battling to stay in business TechCabal	2023	weak	Snippet	techcabal.com/2022/11/19/healthlane-hospitals-obsolete-sta
271	Insurtech aYo debuts micro insurance solutions in Cameroon ITWeb Africa	2023	weak	Snippet	itweb.africa/article/insurtech-ayo-debuts-micro-insurance-
272	JMIR Med Inform 2023 v11 e48256 (search snippet) JMIR Medical Informatics	2023	weak	Snippet	medinform.jmir.org/article/export/end/medinform_v11i1e4825
273	Waspito gets seed extension TechCrunch	2023	moderate	Full	techcrunch.com/2023/11/22/waspito-gets-seed-extension/
274	Digital pathways to expanding private health insurance in Cameroon Nkafu Policy Institute	2024	moderate	Full	nkafu.org/digital-pathways-to-expanding-private-health-ins

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275	openIMIS powers Cameroon's long-term vision for universal health coverage Healthy DEvelopments (BMZ/GIZ)	2024	strong	Full	health.bmz.de/stories/openimis-powers-cameroons-long-term-
276	Cameroon - UHC (openIMIS wiki) openIMIS	2025	unverifiable	Snippet	openimis.atlassian.net/wiki/spaces/OP/pages/3650060289/Cam
277	Cameroun: 3 applications mobiles qui facilitent l'acces aux soins Allo Docteurs	2025	weak	Full	allodocteurs.fr/cameroun-3-applications-mobiles-qui-facili
278	Cameroun: ANTIC, Douala a accueilli le 7e forum sur l'apport des startups du secteur de la sante AllAfrica / Camer.be	2025	moderate	Full	fr.allafrica.com/stories/202601050249.html
279	Africa's Business Heroes 2025 Top 10: Jean Lobe Lobe; Waspito LEAP 2026 headline Africa's Business Heroes; Ecomatin	2026	weak	Snippet	africabusinessheroes.org/en/the-prize/2025/top10/jean-lobe
280	Plan Strategique National de Sante Numerique 2026–2030 (S&E) MINSANTE, Republic of Cameroon	2026	strong	Full	minsante.cm/site/sites/default/files/S%26E_PSNSN%202026-20
281	Waspito: connecting patients to the health ecosystem (search snippet) LinkedIn profile	2026	unverifiable	Snippet	fr.linkedin.com/in/julie-sik
282	HealthTech : quels enjeux pour l'Afrique ? Forbes Afrique (snippet)		weak	Snippet	forbesafrique.com/healthtech-quels-enjeux-pour-lafrique/
283	Isidoctor My Health in the Pocket VC4A		unverifiable	Snippet	vc4a.com/ventures/isidoctor
284	MonDocteur237 Apple App Store		unverifiable	Snippet	apps.apple.com/dk/app/mondocteur237/id6449442828
285	Waspito Launch Africa Ventures		weak	Full	launchafrica.vc/waspito
286	Burundi: hospitals detain indigent patients (HRW) Human Rights Watch	2006	moderate	Snippet	hrw.org/english/docs/2006/09/07/burund14126.htm
287	How do supply-side factors influence informal payments for healthcare: the case of HIV patients in Cameroon International Journal of Health Planning	2006–2007 data	moderate	Snippet	zendy.io/title/10.1002/hpm.2266

#	SOURCE	YEAR	CONFIDENCE	READ	URL
288	Free access to antiretroviral treatment and protection against the risk of catastrophic health expenditure in people living with HIV: evidence from Cameroon BMC Health Services Research (Bousmah et	2014	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC8028721
289	Hospital Detentions for Non-payment of Fees: A Denial of Rights and Dignity Chatham House (Yates, Brookes, Whitaker)	2017	moderate	Snippet	chathamhouse.org/sites/default/files/publications/research
290	Barriers to access and utilization of emergency obstetric care at health facilities in sub-Saharan Africa: a systematic review Systematic Reviews (BioMed Central)	2018	weak	Snippet	ncbi.nlm.nih.gov/pmc/articles/PMC6234634/
291	Paper on User Fees in Cameroon (UNAIDS, Aug 2019) UNAIDS via P4H (Bonnel)	2019	weak	Snippet	p4h.world/app/uploads/2023/02/Paper20on20User20Fees20in20C
292	UHC Day: High health-care costs in Africa continue to push over 150 million into poverty WHO Regional Office for Africa	2019 data	moderate	Snippet	afro.who.int/pt/node/20346
293	Association of Health Care Use and Economic Outcomes After Injury in Cameroon JAMA Network Open (Christie et al.)	2020	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7237963
294	Catastrophic health expenditure in sub-Saharan Africa: systematic review and meta-analysis Bulletin of the WHO (Eze et al.)	2022	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC9047424/
295	Exploring Household Vulnerability to Catastrophic Health Expenditure (Cameroon case) Biomedical Journal of Scientific and Tec	2024	unverifiable	Snippet	biomedres.us/fulltexts/BJSTR.MS.ID.009121.php
296	Debt, detention and the universal healthcare gap: lessons from Fundong, Cameroon (Frontiers in Public Health) Frontiers in Public Health (Tanywe et al	2025	strong	Full	frontiersin.org/journals/public-health/articles/10.3389/fp
297	Deferred recovery of health expenditures for pediatric life-threatening emergencies in a resource-limited setting PLoS One (Penda et al.)	2025	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12140392/
298	Do deferred emergency payment programmes increase use of injury care services in Cameroon? A trauma registry analysis BMJ Global Health (Dissak-Delon et al.)	2025	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC11938216/

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299	Factors influencing direct payment exemption in the user fee elimination project in the Far North of Cameroon Global Health Economics and Sustainabili	2025	weak	Full	accscience.com/journal/GHES/3/2/10.36922/ghes.4078
300	Nkafu Policy Institute article on pricing of medical acts Nkafu Policy Institute	n/a	unverifiable	Snippet	nkafu.org/?p=105905
301	Search results: Ghana NHIS active membership studies BMC / PMC (multiple)	2010	weak	Snippet	pmc.ncbi.nlm.nih.gov/articles/PMC5031274
302	Towards Universal Health Coverage: An Evaluation of Rwanda Mutuelles in Its First Eight Years PLoS One (Lu et al.)	2012	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC3377670
303	Cameroon Health Results-Based Financing Impact Evaluation, endline World Bank Microdata Library	2015	moderate	Snippet	microdata.worldbank.org/index.php/catalog/2047
304	Looking into the performance-based financing black box: evidence from an impact evaluation in the health sector in Cameroon (PRWP 8162) World Bank (de Walque et al.)	2017	strong	Full	econpapers.repec.org/RePEc:wbk:wbrwps:8162
305	Cameroon: Investigate, Distribute Health Fund Human Rights Watch	2020	weak	Snippet	hrw.org/news/2020/06/12/cameroon-investigate-distribute-he
306	Social Protection country profile Cameroon ILO World Social Protection Data	2020	weak	Snippet	social-protection.org/gimi/ShowCountryProfile.action?iso=C
307	Long term financial sustainability of Rwanda's universal health coverage model P4H Network	2023	weak	Snippet	p4h.world/app/uploads/2025/06/Long-term-financial-sustaina
308	Mutuelle suffers Rwf20 billion funding shortage / advance payment model coverage The New Times / KT Press	2023	weak	Snippet	newtimes.co.rw/news/mutuelle-suffers-rwf20-billion-funding
309	18 months of Universal Health Coverage in Cameroon: Transforming Lives and Health Care Management Sciences for Health (RISE pro	2024	moderate	Full	msh.org/story/18-months-of-universal-health-coverage-in-ca
310	Improvements to openIMIS for the Cheque Sante and alignment with phase 1 of the UHC Swiss TPH	2024	moderate	Full	swisstph.ch/en/projects/project-detail/project/improvement
311	Universal Health Coverage Day: Over 3.5 million People Enrolled CRTV	2024	weak	Snippet	crtv.cm/?p=285462

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312	CSU: le Cameroun a depense 318 milliards FCFA en trois ans pour des resultats mitiges (citing Minepat SND30 mid-term review) EcoMatin	2025	moderate	Full	ecomatin.net/couverture-sante-universelle-le-cameroun-a-de
313	Digital Transformation for Social Protection (UN SDGs deck) United Nations / openIMIS	2025	weak	Snippet	sdgs.un.org/sites/default/files/2025-07/Day%201_Digital%20
314	Private health facilities suspend NHIS services over delayed payments Citi Newsroom, 5 April 2025	2025	moderate	Full	citinewsroom.com/2025/04/private-health-facilities-suspend
315	Report says patient verification woes in hospitals deepen under SHA system Eastleigh Voice reporting RUPHA scorecar	2025	moderate	Full	eastleighvoice.co.ke/health/116290/report-says-patient-ver
316	CSU Cameroun, 3 ans : bilan mitige d'une reforme sanitaire ambitieuse Camer.be (Paul Moutila), 30 April 2026	2026	moderate	Full	camer.be/actualites/sante/csu-cameroun-3-ans-bilan-mitige-
317	Lessons from community-based health insurance for scaling universal health coverage in Cameroon Nkafu Policy Institute (Sinsai and Asahn	2026	moderate	Full	nkafu.org/lessons-from-community-based-health-insurance-fo
318	NHIS active membership rises to 70% - Health Minister Graphic Online, 8 Oct 2026	2026	moderate	Full	graphic.com.gh/news/health/nhis-active-membership-rises-to
319	SHA flaws coverage (Nation, Standard, Star, Citizen Digital) Nation Africa / The Standard / Citizen D	2026	weak	Snippet	citizen.digital/news/shif-ting-strategies-biometrics-otps-
320	SHA under fire over delayed payments, system failures The Star (Kenya), 15 May 2026	2026	moderate	Full	the-star.co.ke/news/2026-05-15-sha-under-fire-over-delayed
321	Search coverage across lane queries n/a	2026	unverifiable	Snippet	n/a
322	Effects of a mobile phone short message service on antiretroviral treatment adherence in Kenya (WelTel Kenya1) The Lancet	2010	moderate	Snippet	thelancet.com/journals/lancet/article/PIIS0140-6736(10)619
323	Barriers to the implementation of mobile phone reminders in pediatric HIV care: a pre-trial analysis of the Cameroonian MORE CARE study BMC Health Services Research	2014	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4220055/

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324	Effect of mobile phone reminders on follow-up medical care of children exposed to or infected with HIV in Cameroon (MORE CARE) The Lancet Infectious Diseases	2014	strong	Full	thelancet.com/journals/laninf/article/PIIS1473-3099(14)707
325	Prevalence, predictors and economic consequences of no-shows BMC Health Services Research	2016	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4714455/
326	Chronic staff shortfalls stifle Africa's health systems: WHO study WHO Regional Office for Africa	2018	strong	Full	afro.who.int/news/chronic-staff-shortfalls-stifle-africas-
327	Missed appointments cost the U.S. healthcare system $150B each year Curae (vendor blog)	2018	unverifiable	Snippet	curae.com/2018/03/06/missed-appointments-cost-u-s-healthca
328	Optimizing number and timing of appointment reminders: a randomized trial American Journal of Managed Care	2018	strong	Full	europepmc.org/search?query=%22Optimizing%20Number%20and%20
329	Missed GP appointments cost GBP 216m per year (secondary report of NHS England figures) MDDUS news digest; Public Finance; Pulse	2019	moderate	Snippet	mddus.com/resources/resource-library/news-digest/2019/janu
330	Retention in HIV/AIDS Management Services: is it really poor? The case of Yaounde Central Hospital in Cameroon International Journal of Maternal and Ch	2020	moderate	Snippet	mchandaids.org/retention-in-hiv-aids-management-services-i
331	Predictors of lost to follow up from antiretroviral therapy among adults in sub-Saharan Africa Infectious Diseases of Poverty	2021	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC7981932/
332	Epidemiology of Pediatric Tuberculosis and Factors Associated with Unsuccessful Treatment Outcomes in the Centre Region of Cameroon: A Three-Year Retrospective Cohort Stu Interdisciplinary Perspectives on Infect	2022	moderate	Snippet	ncbi.nlm.nih.gov/pmc/articles/PMC9433292/
333	Behavioural economic interventions to reduce health care appointment non-attendance BMC Health Services Research	2023	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10594857/
334	Powering progress through connectivity: GSMA Mobile Economy Sub-Saharan Africa report GSMA	2023	strong	Full	gsma.com/newsroom/press-release/powering-progress-through-

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335	Testing the Impact of Phone Texting Reminders for Children's Immunization Appointments in Rural Cameroon: Protocol for a Nonrandomized Controlled Trial JMIR Research Protocols	2023	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC10448290/
336	Universal test and treat in Cameroon: a comparative retrospective analysis of mortality and loss to follow-up Pan African Medical Journal	2023	moderate	Full	panafrican-med-journal.com/content/article/45/191/full/
337	Data Protection in Cameroon; Cameroon data protection law overviews DLA Piper; Paul Hastings; Afriwise (seco	2024	moderate	Snippet	dlapiperdataprotection.com/?c=CM&t=law
338	GSMA Mobile Economy Sub-Saharan Africa 2024 (search snippets) GSMA Intelligence	2024	weak	Snippet	gsma.com/mobileeconomy/report/sub-saharan-africa-2024/
339	World Bank Open Data indicators IT.CEL.SETS.P2 and IT.NET.USER.ZS (aggregator snippet only) World Bank via theglobaleconomy.com	2024	weak	Snippet	theglobaleconomy.com/Cameroon/Mobile_phone_subscribers_per
340	Digital 2025: Cameroon DataReportal	2025	strong	Full	datareportal.com/reports/digital-2025-cameroon
341	Evaluating phone call follow-ups in Sub-Saharan Africa: A systematic review and meta-analysis PLOS ONE	2025	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12558458/
342	Hospital Outpatient Activity 2024–25 NHS England Digital	2025	strong	Full	digital.nhs.uk/data-and-information/publications/statistic
343	Systematic review and meta-analysis of no show or non-attendance rates among telehealth and in-person models of care BMC Health Services Research	2025	unverifiable	Full	pmc.ncbi.nlm.nih.gov/articles/PMC12063363/
344	National Digital Health Strategic Plan 2026–2030 (implementation framework, English) Ministry of Public Health, Cameroon	2026	strong	Full	minsante.cm/site/sites/default/files/IF_S%26E_NDHSP%202026
345	Global strategy on human resources for health: Workforce 2030 World Health Organization	2030	strong	Full	who.int/teams/health-workforce/global-strategy
346	A systematic review and meta-analysis of appointment reminders for enhancing hospital attendance Journal of Hospital Management and Healt		moderate	Snippet	jhmhp.amegroups.org/article/view/10215/html

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347	Absenteeism among public sector health professionals in Cameroon (APHA 138th meeting abstract) American Public Health Association	2008 data, conference abstract	weak	Snippet	apha.confex.com/apha/138am/webprogram/Paper227490.html
348	A systematic review of task-shifting for HIV treatment and care in Africa Human Resources for Health (Callaghan, F	2010	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC2873343/
349	Cameroon or Canada? Poorly paid doctors and nurses are choosing to leave. That's common in Africa ABC News / Associated Press	2011–2024	weak	Snippet	abcnews.go.com/Health/wireStory/cameroon-canada-poorly-pai
350	Surgical capacity in Fako Division, Cameroon (search snippet via Springer World Journal of Surgery) World Journal of Surgery	2013	weak	Snippet	link.springer.com/10.1186/s12913-015-1147-y
351	Global surgery 2030: evidence and solutions for achieving health, welfare, and economic development The Lancet Commission on Global Surgery	2015	moderate	Snippet	staging19surgery.ucsf.edu/sites/default/files/umbraco/medi
352	A cross-sectional survey of emergency and essential surgical care capacity among hospitals with high trauma burden in a Central African country BMC Health Services Research (Kouo-Ngamb	2015 (data 2009)	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC4619297
353	Task-shifting in sub-Saharan Africa: qualitative systematic review of health workers' experiences (Mijovic, McKnight, English) Journal of Clinical Nursing (via IDEAS/R	2016	weak	Snippet	ideas.repec.org/a/wly/jocnur/v25y2016i15-16p2083-2100.html
354	WHO issues two reports detailing global problem of substandard, falsified medicines Intellectual Property Watch	2017	moderate	Full	ip-watch.org/2017/11/28/issues-two-reports-detailing-globa
355	CENAME receives three anti-TB drug containers to curb shortage StopBlaBlaCam	2020	weak	Snippet	stopblablacam.com/culture-and-society/0303-3414-cename-rec
356	Does performance-based financing curb stock-outs of essential medicines? Results from a randomised controlled trial in Cameroon Tropical Medicine & International He	2020	moderate	Snippet	zendy.io/title/10.1111/tmi.13447
357	Availability, prices and affordability of selected antibiotics and medicines against non-communicable diseases in western Cameroon and northeast DR Congo PLoS ONE (Schafermann et al.)	2020 (data 2017–2018)	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC6946586

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358	Identification of Substandard and Falsified Medicines: Influence of Different Tolerance Limits and Use of Authenticity Inquiries American Journal of Tropical Medicine an	2021	moderate	Full	pmc.ncbi.nlm.nih.gov/articles/PMC8103440
359	Performance-based financing and essential medicine stock-outs in Cameroon (IRD Horizon document 010083005) IRD Horizon repository	2021	weak	Snippet	horizon.documentation.ird.fr/exl-doc/pleins_textes/2021-10
360	Poor-Quality Medicines in Cameroon: A Critical Review American Journal of Tropical Medicine an	2021	strong	Full	ajtmh.org/downloadpdf/journals/tpmd/105/2/article-p284.xml
361	WHO African Region status report on blood availability, safety and quality WHO Regional Office for Africa	2022 report (2016 data)	moderate	Full	afro.who.int/node/16779
362	World Blood Donor Day: Harmonization of Blood Collection System Among Major Reforms CRTV	2022–2026	weak	Snippet	actu.crtv.cm/?p=311084
363	37 of 55 countries facing health worker shortages in Africa: WHO Down To Earth (reporting WHO list of 14	2023	moderate	Full	downtoearth.org.in/world/37-of-55-countries-facing-health-
364	WHO health workforce support and safeguards list 2023 World Health Organization	2023	strong	Full	who.int/publications/i/item/9789240069787
365	Cameroun: l'Etat et les particuliers enfoncent la CENAME dans la crise avec plus de 14 milliards de dettes Ecomatin	2023 (data 2021)	moderate	Full	ecomatin.net/cameroun-letat-et-les-particuliers-enfoncent-
366	Prevalence of Poor Quality Ciprofloxacin and Metronidazole Tablets in Three Cities in Cameroon American Journal of Tropical Medicine an	2023 (sampled Dec 2019 to Dec 2020)	strong	Full	pmc.ncbi.nlm.nih.gov/articles/PMC9896317
367	Cameroon doctors flee to Europe, North America for lucrative jobs Voice of America	2024	moderate	Full	voanews.com/a/cameroon-doctors-flee-to-europe-north-americ
368	Human resource challenges in health systems: evidence from 10 African countries Health Policy and Planning (Sheffel et a	2024	strong	Full	thedocs.worldbank.org/en/doc/22a61eae728ba4af1f2dcdd7d5909

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369	Identifying opportunities for global surgery in Cameroon: an analysis of existing health policies and events Pan African Medical Journal (Takoutsing	2024	moderate	Full	panafrican-med-journal.com/content/article/47/143/full
370	Thousands of doctors leaving Cameroon for Europe, North America: health ministry Gazette NGR (citing VOA)	2024	weak	Full	gazettengr.com/thousands-of-doctors-leaving-cameroon-for-e
371	Penurie d'insuline et vaccin: l'Ordre des pharmaciens interpelle le Premier ministre Ecomatin	2024–2025	unverifiable	Snippet	ecomatin.net/penurie-dinsuline-et-vaccin-lordre-des-pharma
372	Babies' deaths in Cameroon show how US aid cuts curtail malaria fight Reuters via TimesLIVE	2025	moderate	Full	timeslive.co.za/news/africa/2025-10-02-watch-babies-deaths
373	The Benkadi blood project ISBT Transfusion Today (July 2025)	2025	moderate	Full	isbtweb.foleon.com/transfusion-today/transfusion-today-jul
374	Blood safety and availability World Health Organization fact sheet	2025 fact sheet (data 2023)	strong	Full	who.int/news-room/fact-sheets/detail/blood-safety-and-avai
375	Blood Donation: Need For Urgent Collective Action Cameroon Tribune (9 April 2026)	2026	moderate	Full	cameroon-tribune.cm/article.html/75915/en.html/details_2
376	Medical oxygen availability in Cameroon hospitals (IRD Horizon document 010095831) IRD Horizon repository	2026 (document dated 2026-01)	weak	Snippet	horizon.documentation.ird.fr/exl-doc/pleins_textes/2026-01
377	Factors associated with the readiness assessment of health facility services in Yaounde, Cameroon Sciety / preprint (SARA-adapted)	2026 preprint	weak	Snippet	sciety.org/articles/activity/10.64898/2026.06.30.26356973
378	Violence continues in Cameroon, access to healthcare remains seriously limited Medecins Sans Frontieres	not confirmed	weak	Snippet	msf.org/violence-continues-cameroon-access-healthcare-rema
379	Absenteeism of Healthcare Workers in Primary Healthcare in Sub-Saharan Africa: A Scoping Review Klootwijk et al., Int J Health Plann Man	not stated on page (DOI 10.1002/hpm.3890)	moderate	Full	citedrive.com/en/discovery/absenteeism-of-healthcare-worke